Greyhound Blood Bank LIS

Administrator Guide

Hospital transfusion-service laboratory information system

Compliance-supporting transfusion-service software. Not FDA-cleared, AABB-accredited, or CAP-certified by publication of this guide.

This guide describes the screens in the application. Facility procedures, medical-director policy, and the validation record still govern clinical use. Where a behavior depends on a local decision, the topic says so.

Figures show the development training database. They are not patient records from a clinical service.

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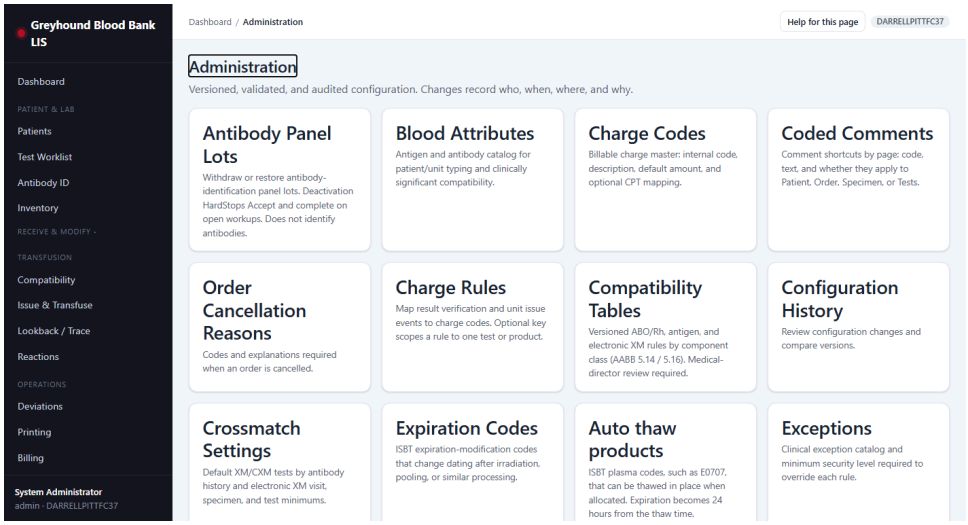
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Getting started

Administration home

Configuration is versioned and audited. This page is the index of every catalog, rule, and interface setting.



Administration is the index of configuration. Each card opens one catalog or rule set. You only see cards your permissions allow. Viewing configuration is not the same as editing it. Edit permission is separate, and privilege changes to users require a reason.

How a change is stored

Edits do not overwrite history. Saving records who changed the row, when, at which workstation, and why. **Configuration History** can compare versions. The clinical audit trail also receives the event. You cannot delete a historical version to undo a mistake. Enter a new version that corrects it, with a reason that says what was wrong.

> Facility policy. Turning on electronic crossmatch, changing a specimen window, or editing a compatibility table changes what the bench is allowed to do. Make those changes under your change-control procedure, not as a casual edit during a shift.

Where to go next

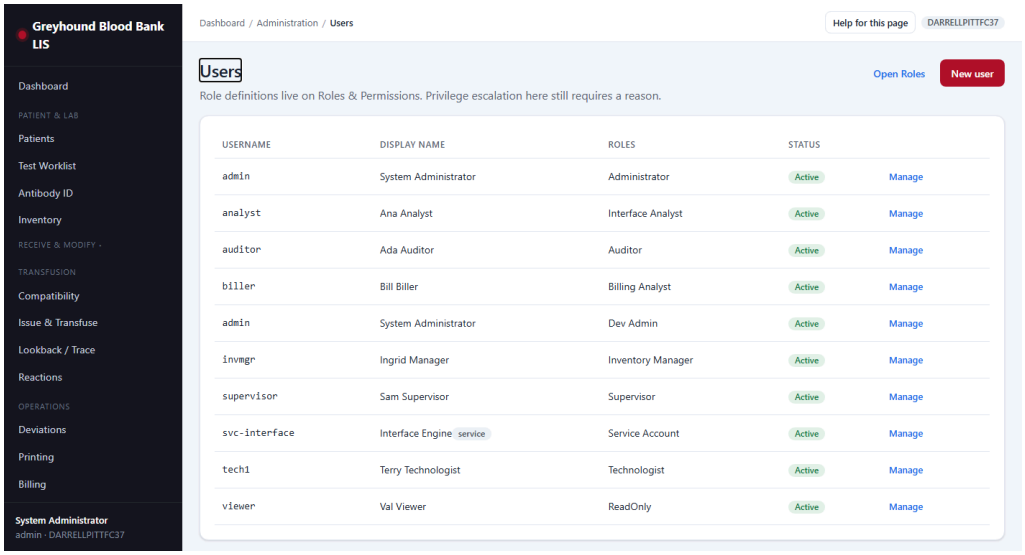
User accounts and roles are under Directory and audit. Test, product, and location catalogs are under Catalog. Safety behavior is under Rules and safety. Suppliers, locations, and ISBT codes are under Inventory and ISBT. HL7, instruments, and printers are under Interfaces. Report definitions are the report designer.

The Clinical User Guide describes what technologists see after you save. Read that topic before you change a rule that issues blood, verifies a result, or captures a charge.

Directory and audit

Users

Create accounts, assign roles, and lock or deactivate a user. Privilege increases require a reason.



Users lists accounts. **New user** creates one. **Manage** on a row edits it. Roles themselves are defined on Roles. This screen assigns those roles to a person.

What to set

The username is what the person types at sign-in. The display name is what other screens show. A service account is for an interface, not for a person at a bench. Do not give a shared bench password to a service account, and do not let people sign in as the service account to bypass their own role.

Status is active, locked, or inactive. Lock is what you do after five failed passwords or when someone should be stopped immediately. Inactive is for someone who has left. Both end the ability to sign in. An open session is revoked when the account is locked or deactivated.

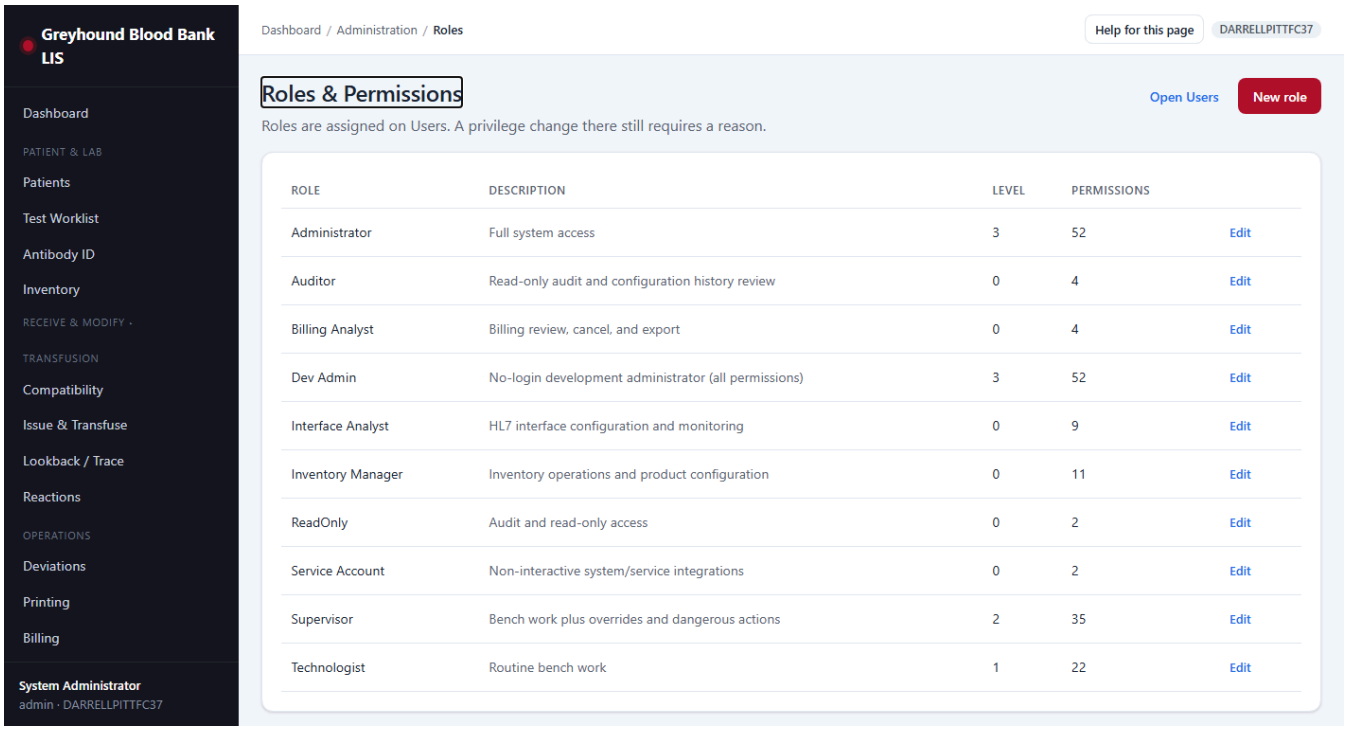
Privilege changes

Adding a role that grants more permission, especially emergency release, result correction, user administration, or configuration edit, requires a reason. The reason is audited. Removing a role should also say why. After you change roles, tell the person to sign out and sign in. The session they already have keeps the old permissions until it ends.

> Warning. An administrator account can change every clinical rule. Do not use it for routine bench work. The audit trail will attribute those results and issues to the administrator.

Roles and permissions

A role is a named set of permission codes plus a security level used for exception overrides.



Roles defines what a job can do. The application checks permission codes, not the role name. Renaming a role does not change access. Changing the codes on it does.

Security level

Each role has a security level. Exception overrides in the exception catalog require the user's role to meet a minimum level as well as the permission code. A technologist role at a lower level cannot override a warning that is reserved for a supervisor level, even if someone copies a permission code onto that role by mistake. Check both the code and the level when an override "should work" and does not.

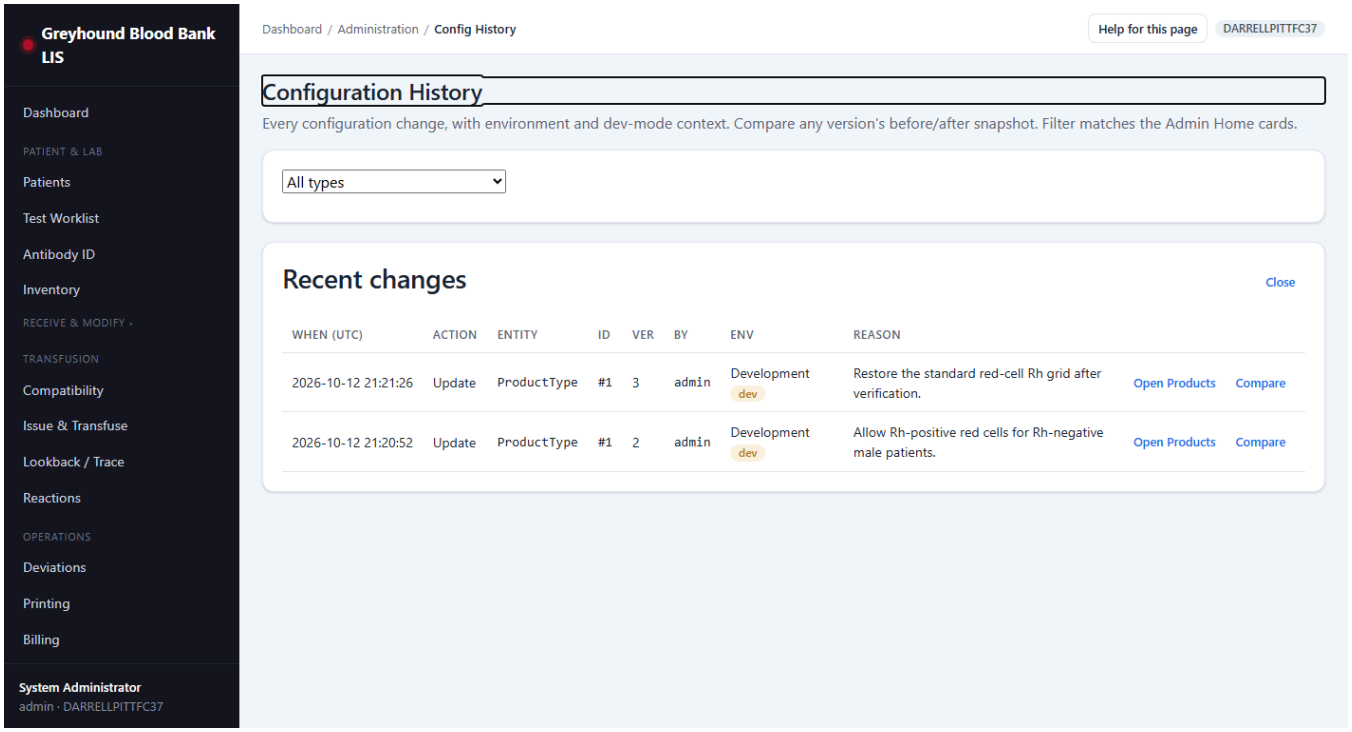
Seeded roles

A training database includes Administrator, Supervisor, Technologist, Read only, Interface analyst, Inventory manager, Billing analyst, Auditor, and a service account. Treat those as examples. Your facility's roles should match your procedure. Do not grant **issue.emergency-release** to every technologist because the supervisor is at lunch. Emergency release is a separate permission from ordinary issue.

Open Users to see who holds a role. Changing the role changes everyone assigned to it the next time they sign in.

Configuration history

Compare versions of configuration changes and see who saved them.



Configuration History lists changes to catalogs and rules. Open a change to compare the previous version with the new one. You need permission to review the configuration audit.

Use this screen when the bench reports that a rule "changed overnight." The history shows the

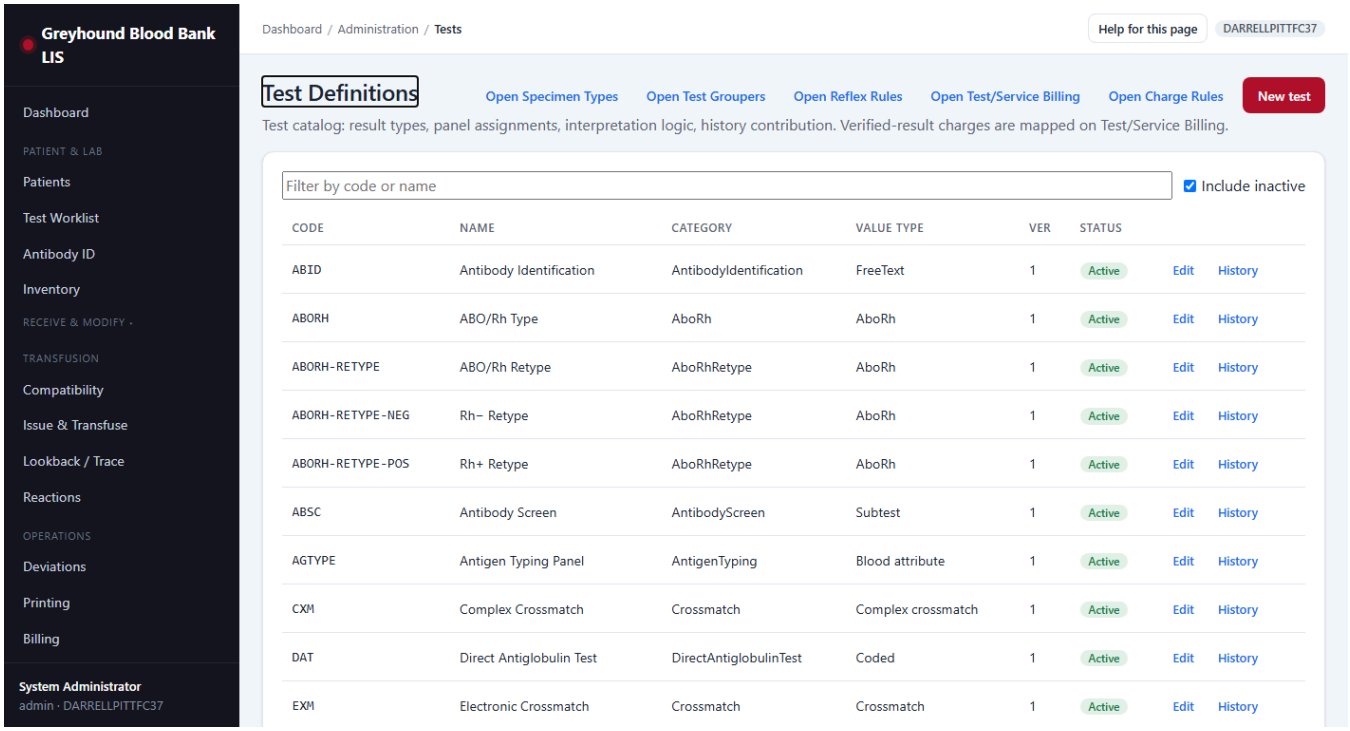
parameter, the old value, the new value, the user, and the reason. It does not offer a one-click revert that hides the bad version. If the new value is wrong, open the catalog it came from and save a corrected value with a reason that points at this history row.

Clinical actions (issues, results, logins) are on the Audit trail in the clinical guide. Configuration history is the narrower record of setup changes.

Catalog

Test definitions

Define each test's result type, panel, interpretation, and whether it contributes to blood-type or antibody history.



Test Definitions is the catalog the order entry and the worklist use. A test that is not here cannot be ordered. Each definition says how the result is recorded (a choice list, a graded reaction, free text), which panel subtests it includes, and how an interpretation is derived.

History

Mark the tests that write patient ABO, Rh, antibody screen, or antibody identification into history. Those are the tests the issue gate and electronic crossmatch read. A test you forget to mark will verify on the worklist and still leave the chart without a blood type. A test you mark by mistake can overwrite history with a value that was never meant to be a type.

Interpretation

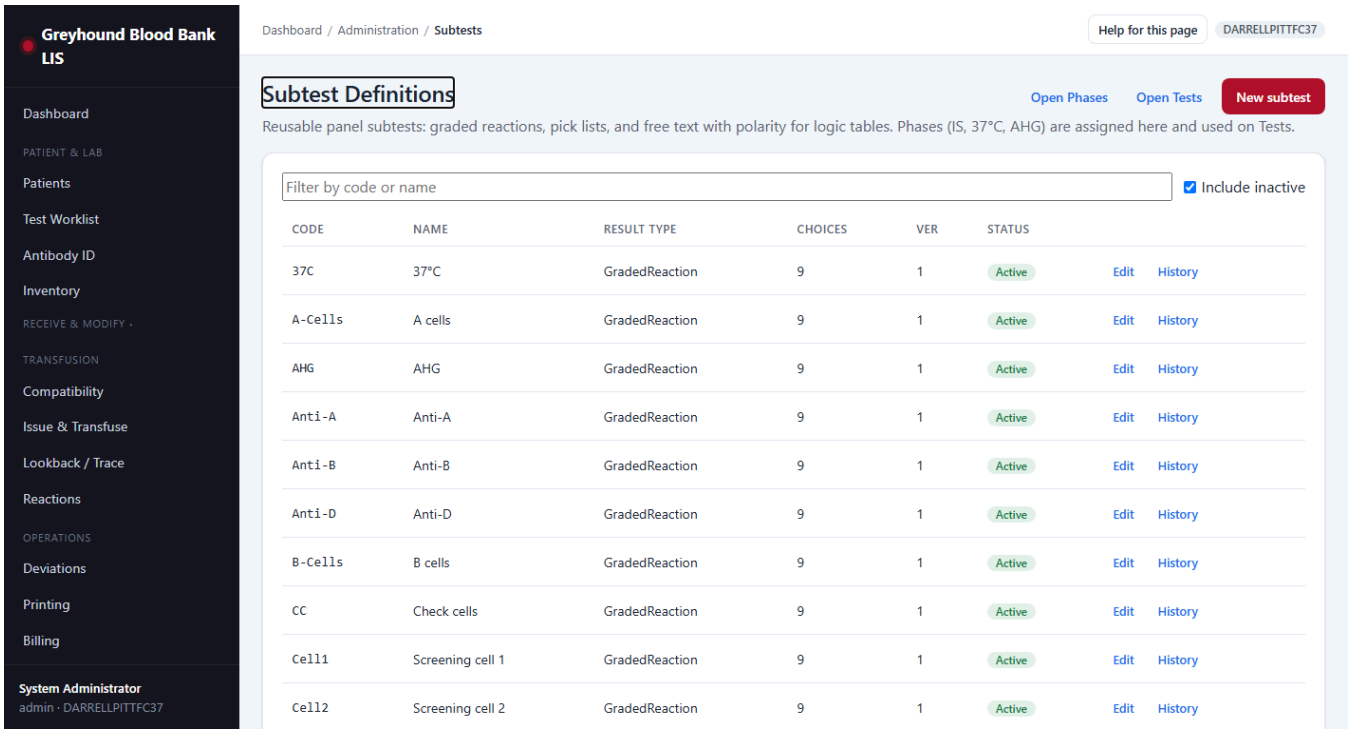
Panel interpretation uses the subtests and the logic table on the definition. Changing the logic changes what "positive" means for every future verification. It does not rewrite verified history. Coordinate the change with the medical director and record the reason.

Inactivating a test stops new orders. It does not remove historical results. Prefer inactive over deleting a code that already appears on patient records.

Billing for a verified test is mapped on Test and service billing, not on this page.

Subtests

Define reusable panel rows: graded reactions, pick lists, or free text, with a polarity the logic tables can read.



Subtests are the rows inside a panel: each cell or each phase reading that a test definition can include. A subtest has a code, a result style (graded reaction, pick list, or free text), and a polarity so interpretation tables know which answers count as positive.

Create a subtest once and attach it to the tests that need it. Editing the meaning of a graded reaction changes every test that uses that subtest. If only one test should change, create a new subtest rather than redefining a shared one.

Phases assigned to panel subtests are maintained on Phases. The subtest says what is being read. The phase says when it was read.

Test groupers

Bundle member tests so ordering a grouper, such as a type and screen, expands into the individual test lines.

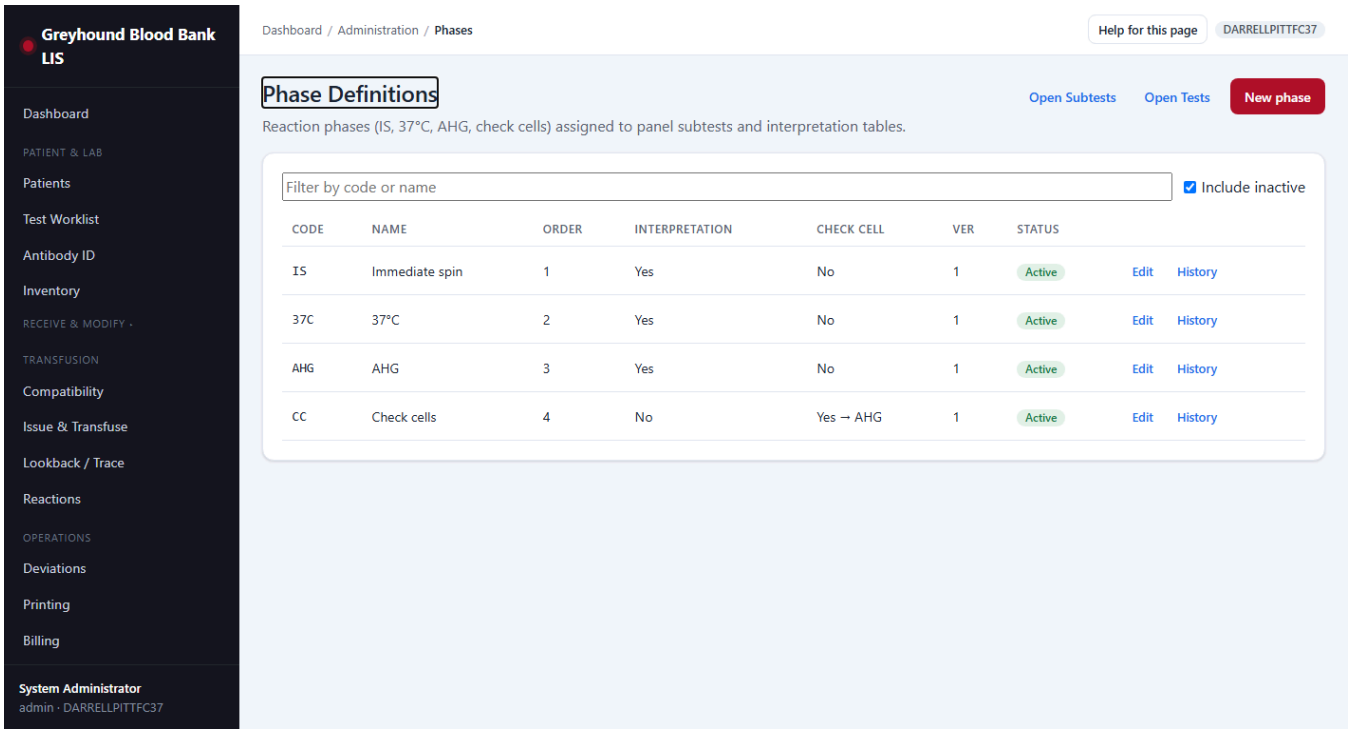
A **test grouper** is an orderable bundle. The technologist orders the grouper. The system expands it into the member tests on the same order and specimen. A type and screen is the usual example. The members are test definitions. The grouper does not have its own result.

Changing the members changes the next order. It does not add or remove tests from orders already placed. Retire a grouper by inactivating it when the code has been used, so old orders still display the name.

Do not create two groupers with the same clinical meaning and different codes unless the billing or the interface truly needs both. Interface maps often send one code. A second grouper will not receive those orders.

Phases

Name the reaction phases, such as immediate spin, 37°C, AHG, and check cells, used by panel subtests.



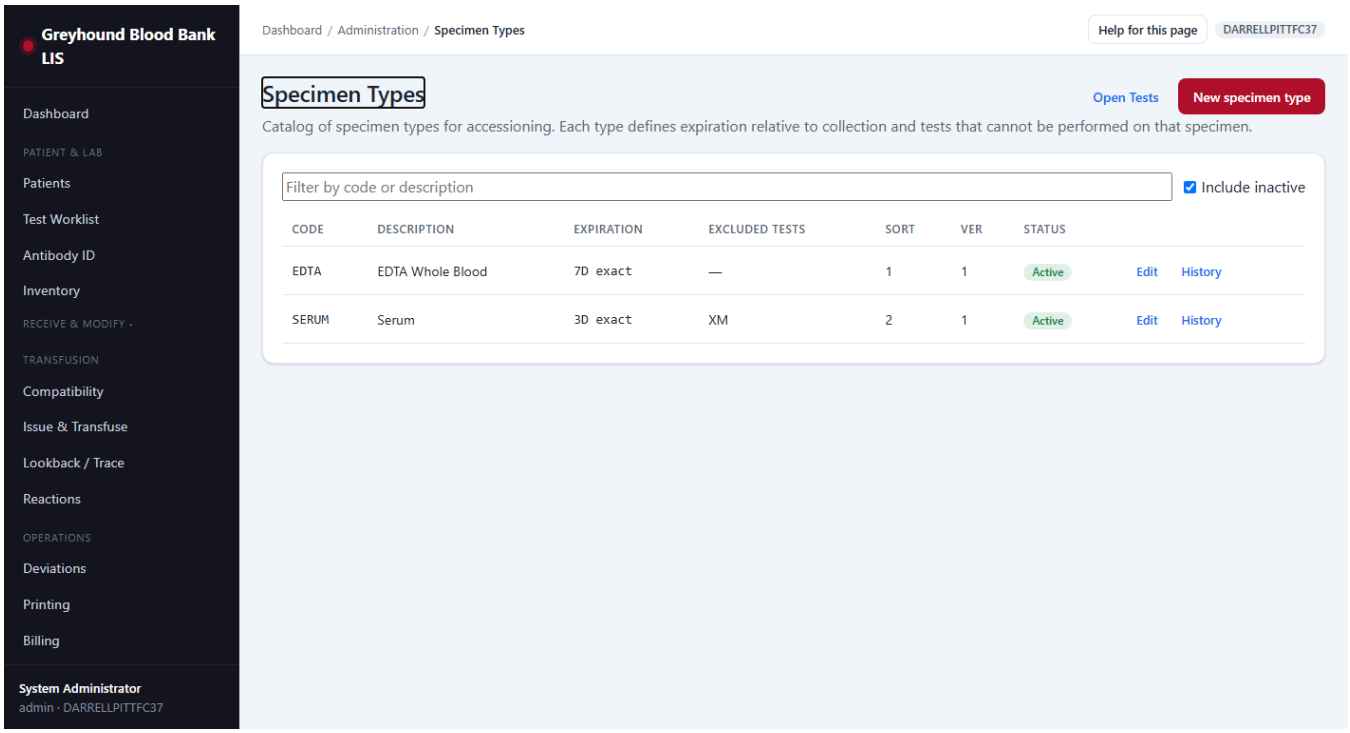
Phases are the reading stages on a panel: immediate spin, 37°C, AHG, check cells, and any stage your procedure adds. Subtests and interpretation tables reference these rows.

Add a phase when the procedure adds a reading. Do not rename a phase to mean a different stage if historical panels used the old name. Inactivate the old row and add the new one so old workups still display the phase they were read at.

The antibody-identification screen and the crossmatch reaction fields show phases in the order defined here. Put them in the order the bench reads them.

Specimen types

Define specimen types used at accession, including tests that cannot be performed on that type.



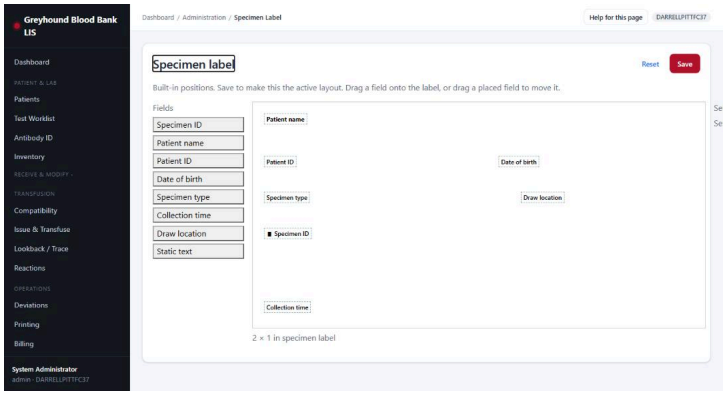
Specimen Types is the list accessioning offers. Each type can exclude tests that must not be ordered on it. The exclusion is enforced when the order is placed or the specimen is linked. It is not a warning the bench is expected to remember.

Add a type only when collection and testing rules differ. A duplicate type with a slightly different name splits the worklist and the interface map. If an interface sends a specimen-type code, that code must match a row here or the message fails translation.

Inactivate a type that should no longer be collected. Historical specimens keep the type they were drawn as.

Specimen label layout

Place patient, specimen, and barcode fields on the 2 by 1 inch specimen label. One layout is active.



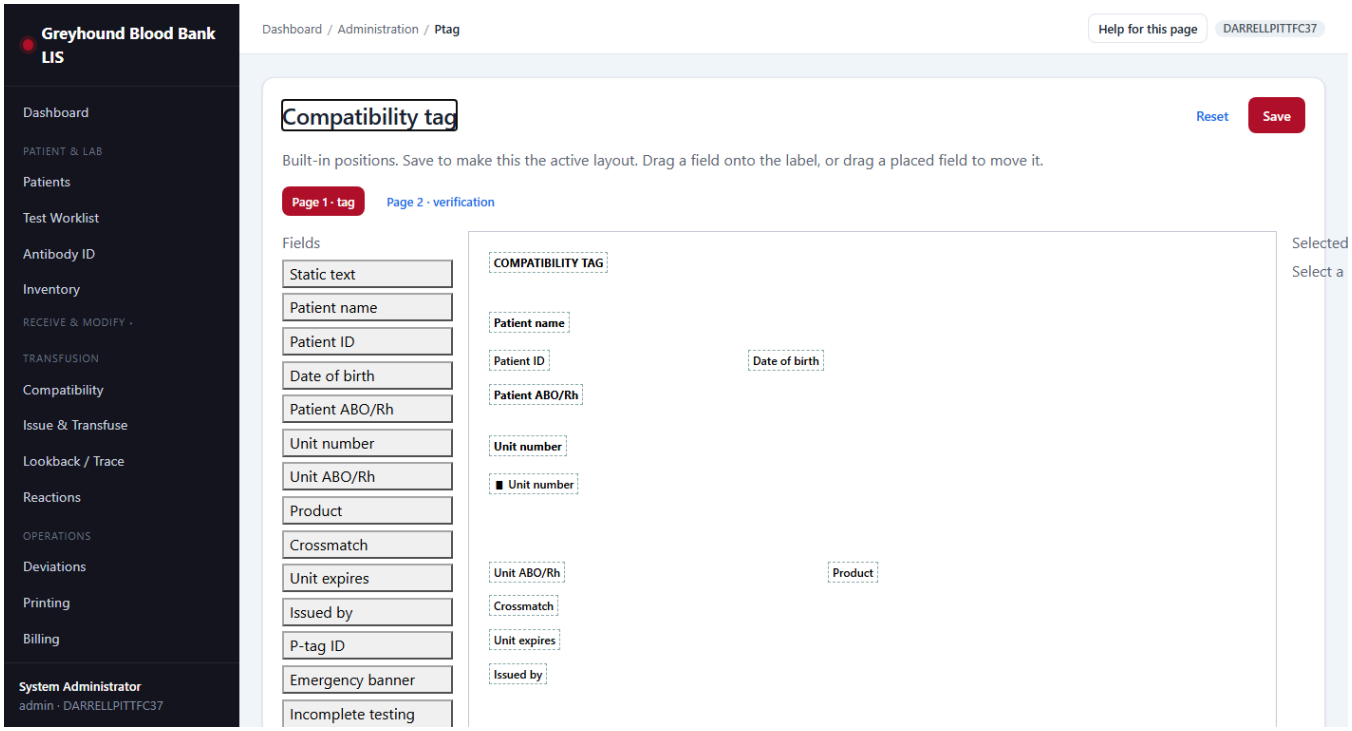
Specimen Label is the layout designer for the 2×1 inch specimen label. Drag the patient, specimen, and barcode fields. One layout is active. The next specimen label printed anywhere in the application uses it.

Print a sample to a test printer before you activate a layout. A field that fits on the screen can still collide with the barcode on the printer. The barcode has to scan. If it does not, move the other fields. Do not shrink the barcode below what your scanners read.

This page does not choose the printer. Printers are on Printers. Technologists pick the printer at print time. They cannot move fields.

P-tag layout

Design the compatibility tag and the fold-under verification form. One layout is active.



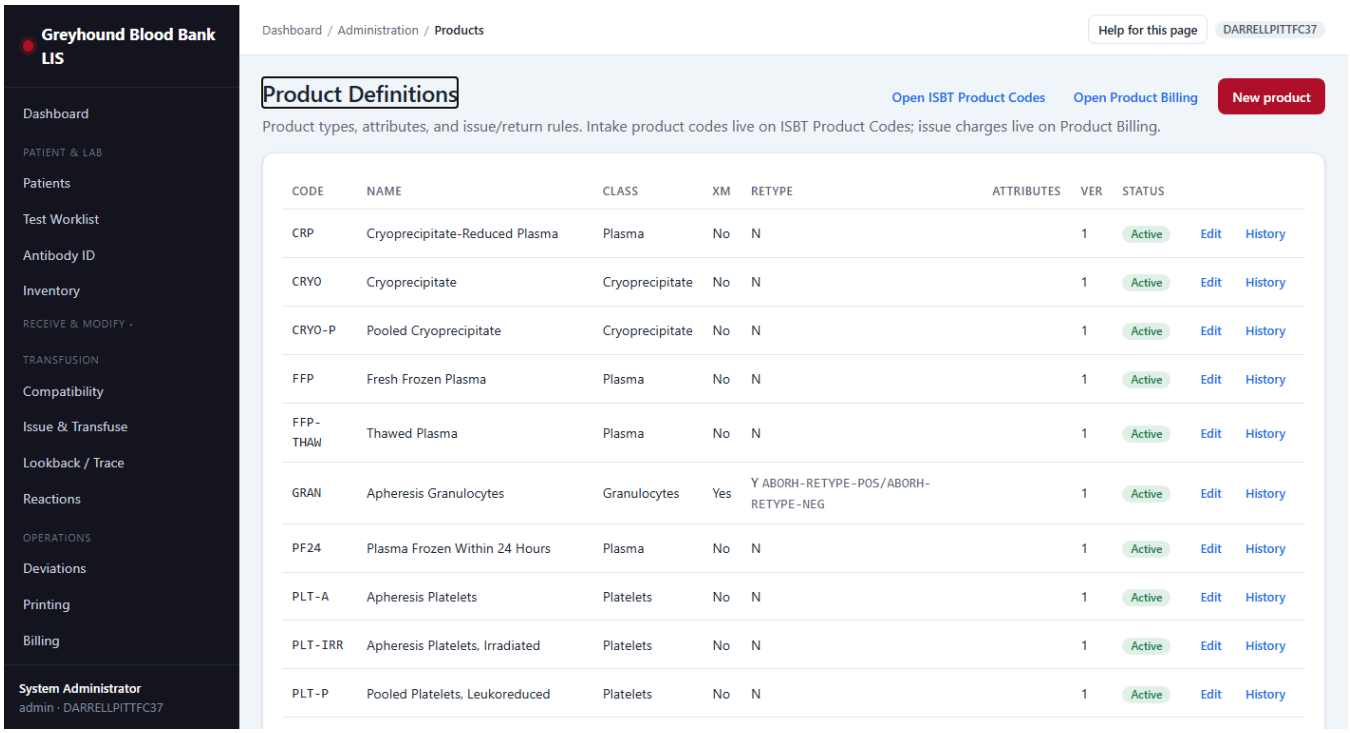
P-tag designs the compatibility tag issued with a unit and the fold-under form used to verify it. One layout is active. Issue and Printing both use that layout.

Include the patient identifiers, the unit identifiers, and the crossmatch information your procedure requires the ward to see. If facility policy requires a scanned P-tag id before issue, the printed tag must carry that id as a barcode the issue form can read. A tag that looks right but will not scan stops issue at the desk.

Activate a changed layout only after a test print. Tags already printed keep the layout they were printed with. They are not reprinted automatically.

Products

Define product types, attributes such as irradiated or leukoreduced, and the issue and return rules for each.



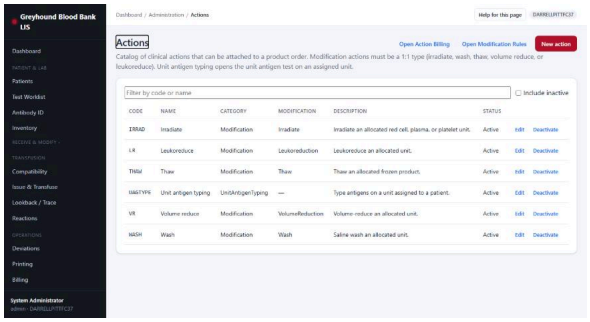
Products describes what a unit is allowed to do: which attributes it can carry (irradiated, leukoreduced, and the others you define) and the rules for issue and return. The ISBT product code catalog is separate. This page is the local product behavior those codes map onto.

Return rules decide whether a unit that comes back unused can be returned to available stock, and under what temperature and seal checks. Loosening a return rule lets the issue desk restock a unit your procedure says must be discarded. Tighten a rule only when the medical director has agreed, and say so in the reason.

Attributes you add here become available on modification rules and on the unit chart. An attribute that is not defined cannot be recorded, so the special-requirement check cannot see it.

Actions

Define modification actions that can be attached to a product order, such as irradiate, wash, thaw, volume reduce, and leukoreduce.



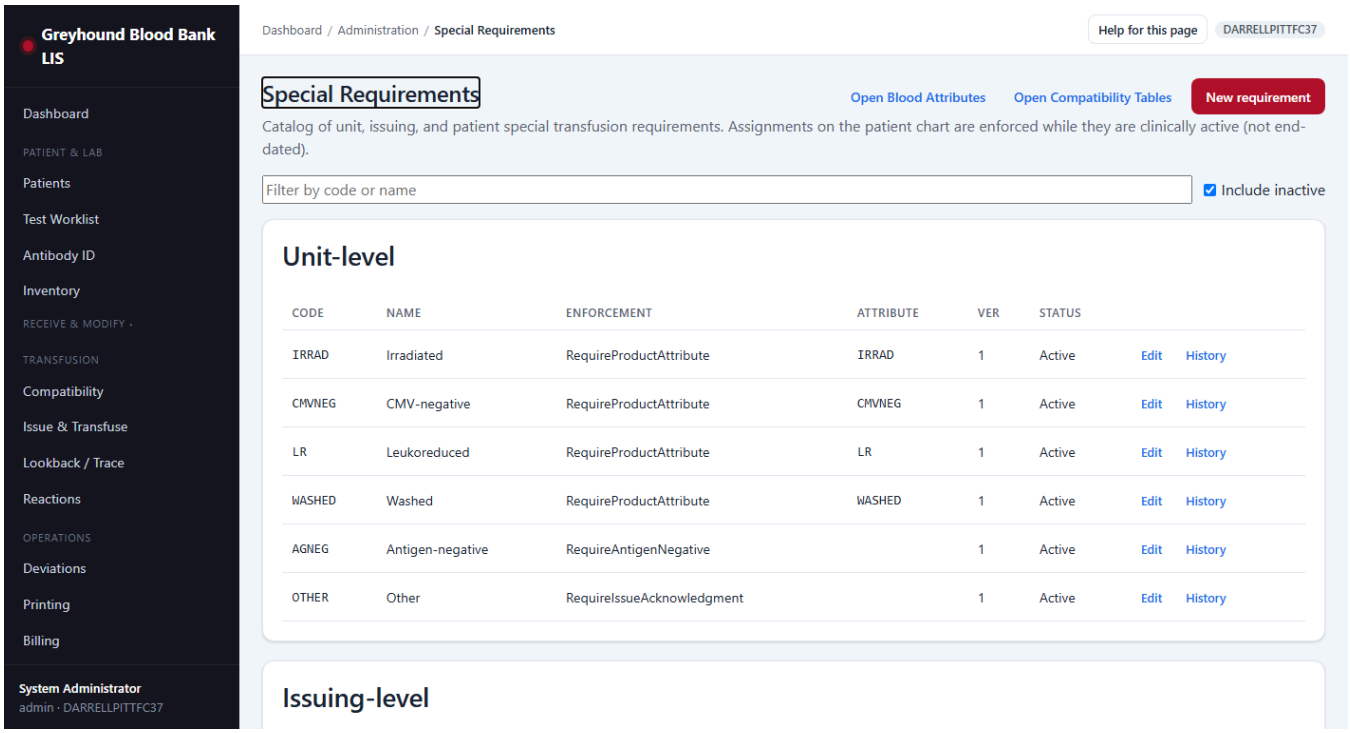
Actions are the modifications a product order can request: irradiate, wash, thaw, volume reduce, leukoreduce. The order stores the request. Modify products is where the bench completes it. The modification rule says which source products can become which result.

Add an action only when both the order form and a modification rule will use it. An action with no rule cannot be completed. An action with no billing map will complete without a charge. Map the charge on Action billing if the completed action is billable.

Inactivate an action to stop new orders from requesting it. Open orders that already requested it still show the request.

Special requirements

Define requirement types that can be applied to a patient, a unit, or an issue, and that the issue gate enforces while they are active.



Special Requirements is the catalog of requirement types: irradiated, antigen-negative specificities, and the other constraints your laboratory uses. The patient chart turns a type on or off for a person. An end date makes it inactive. While it is active, selection and issue enforce it.

Create the type here before the bench can apply it. Wording matters because the issue alert repeats it. Say what must be true of the unit, not a vague precaution.

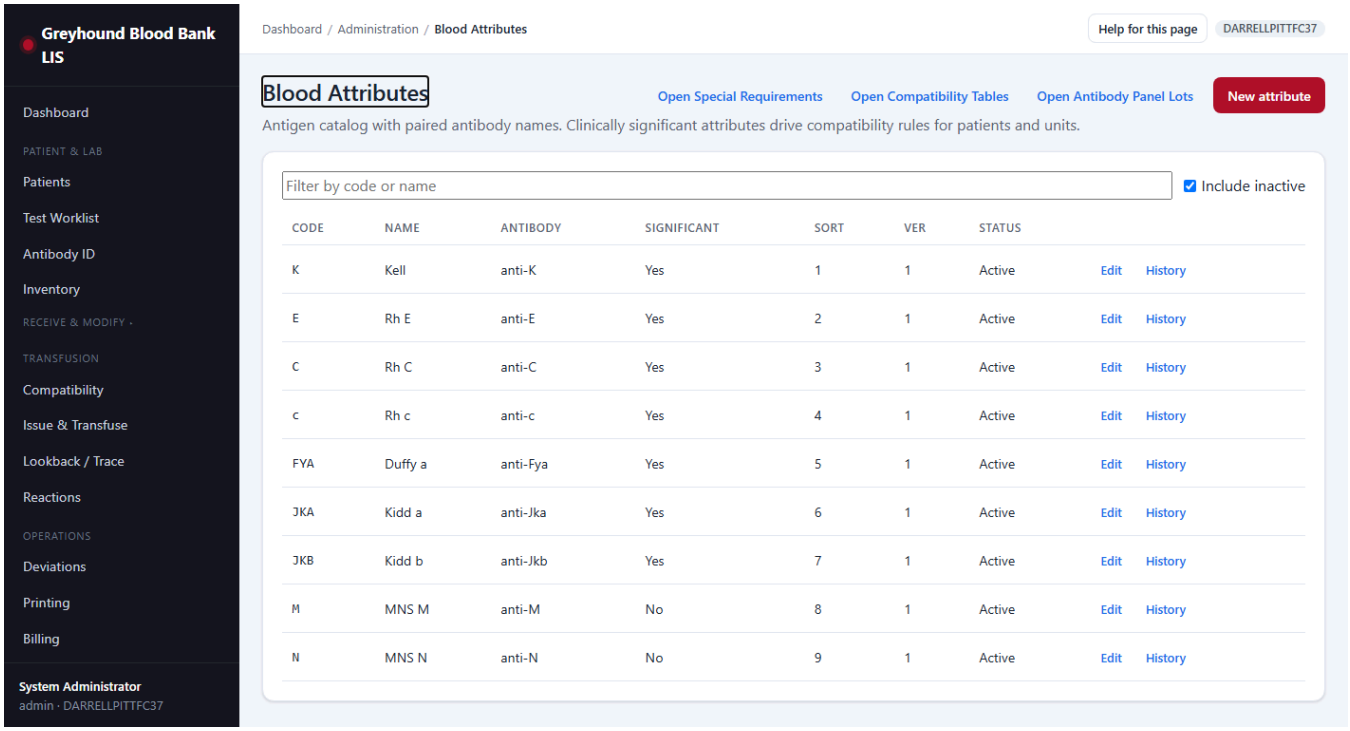
> Facility policy. Which patients require irradiation or antigen-negative units is a medical-director policy. This catalog only makes the requirement enforceable. It does not decide who needs it.

Antigen-negative types depend on the blood-attribute catalog and on unit typing. A requirement for an

antigen you cannot type will warn or stop at issue until a typed unit is chosen.

Blood attributes

Maintain the antigen and antibody catalog used for patient history, unit typing, and compatibility.



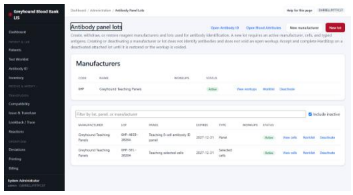
Blood Attributes is the antigen and antibody list. Patient antibody history, unit antigen typing, and compatibility rules reference these codes. A specificity that is not in the catalog cannot be posted from an identification workup and cannot be required as antigen-negative.

Mark clinical significance the way your medical director defines it. That flag is what compatibility uses when it decides an antibody must be honored. Changing significance changes future crossmatch and issue behavior. It is a rule change, not a spelling fix.

Do not create a second code for an antibody you already have because a panel used a synonym. Add the synonym if the screen allows one, or map it in the interface translation table. Two codes split the history and can make electronic crossmatch look at only one of them.

Antibody panel lots

Withdraw or restore antibody-identification panel lots. A withdrawn lot cannot be accepted or completed.



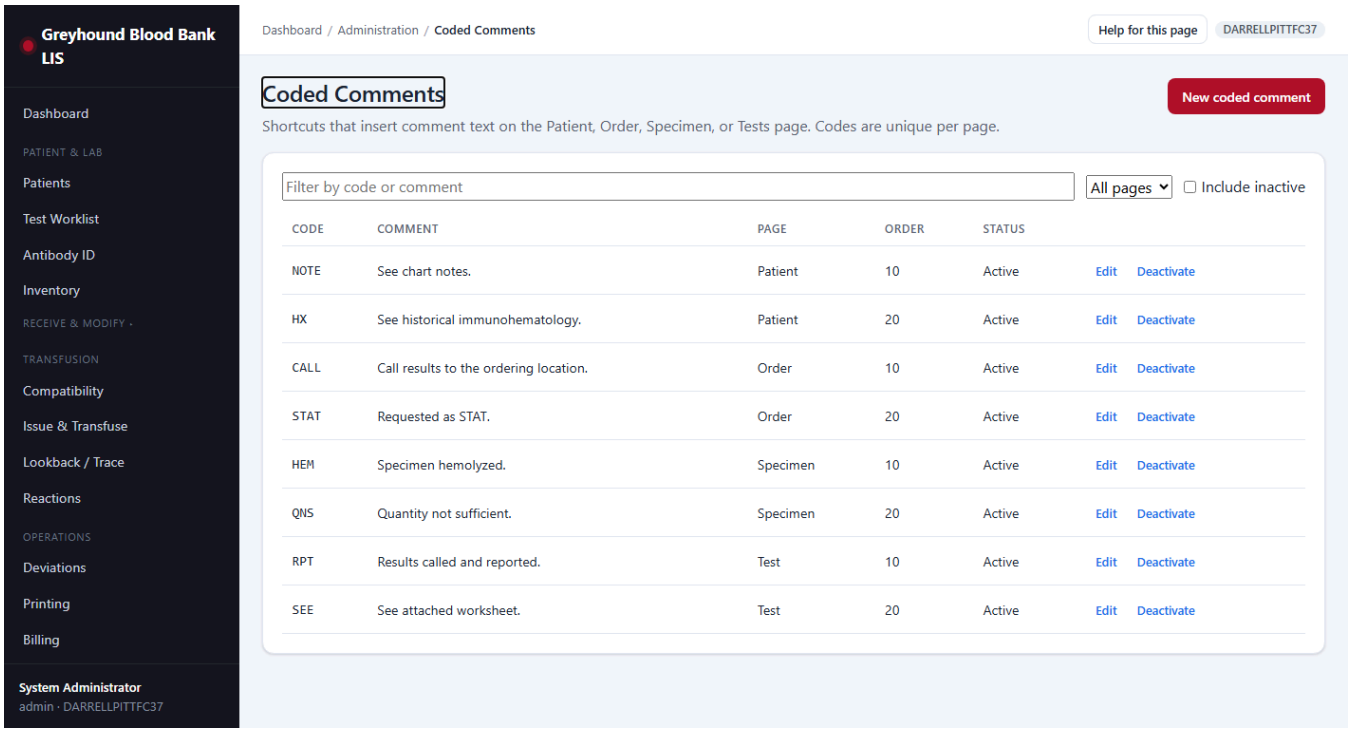
Antibody Panel Lots tracks the panel lots the identification workup can use. Withdraw a lot when it is expired, contaminated, or recalled. A withdrawn lot cannot be accepted on a new workup and cannot be completed. Restore a lot only if the withdrawal was a mistake and the lot is still valid to result.

Withdrawing a lot does not identify antibodies and does not close workups that were already completed on that lot. Open workups on a withdrawn lot have to move to a valid lot or be closed without using the withdrawn one.

The lot's antigram is what **Assist** compares. A wrong antigram produces wrong suggestions. Assist still does not identify the antibody. Correct the antigram before the bench uses the lot, and record why.

Coded comments

Define comment shortcuts by screen, for patient, order, specimen, or test comments.



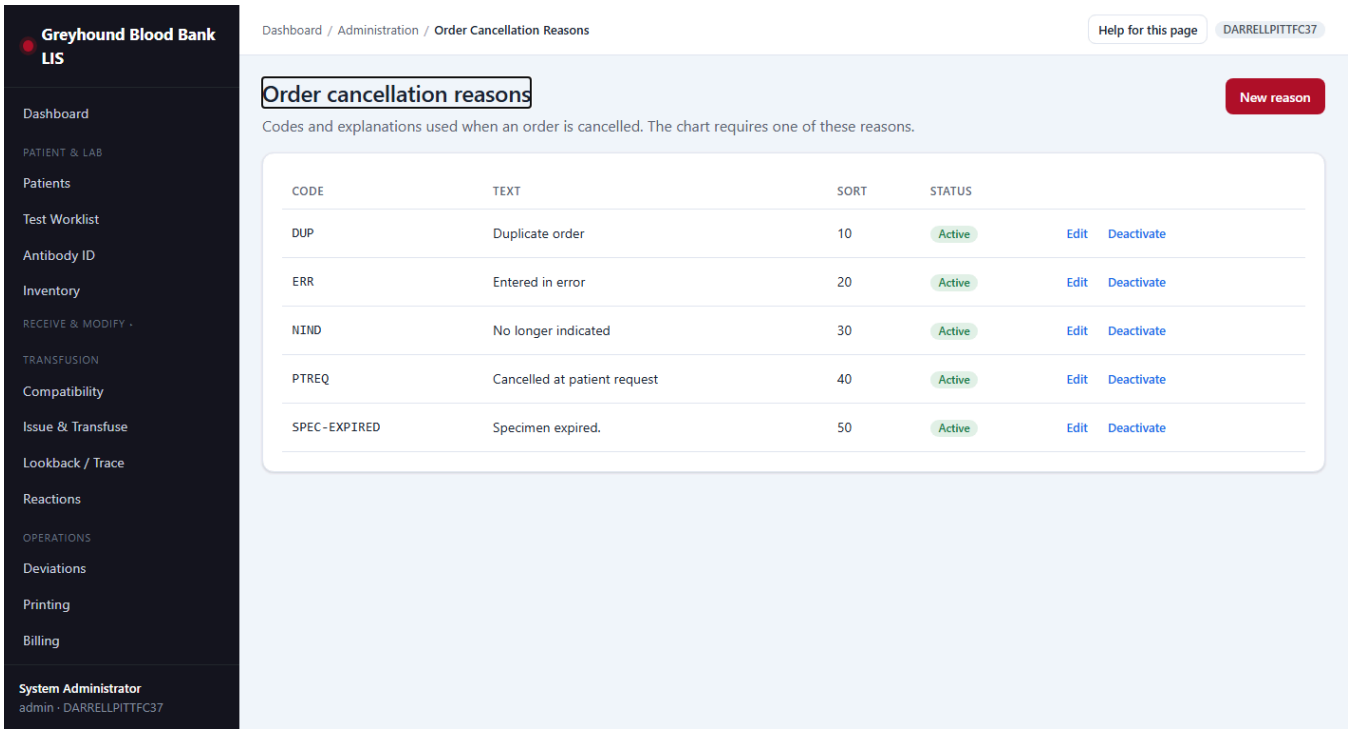
Coded Comments are the shortcuts that appear in comment fields. Each row has a code, the text it inserts, and the pages it belongs on: Patient, Order, Specimen, or Tests. A code defined for specimens does not appear on the patient comment.

Write the text as a complete comment. The bench inserts it and can add to it. Changing the text changes what the shortcut inserts next time. It does not rewrite comments already saved on charts.

Retire a code that your procedure no longer wants by inactivating it. Reusing the same code for a different sentence later will confuse anyone who learned the old expansion.

Order cancellation reasons

Define the reasons a user must pick when cancelling an order.



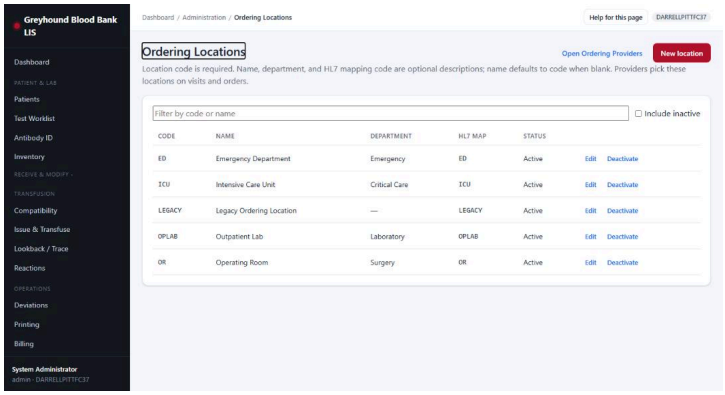
Order Cancellation Reasons is the list the order screen requires. Cancellation cannot be saved without a code. Add a reason for each situation your procedure distinguishes, such as duplicate order, patient discharged, or collected on the wrong patient. The code and the explanation are both stored on the order.

Do not add a single reason called "Other" and expect the bench to explain in a comment if your procedure needs the reason to be reportable. The code is what reports and the audit trail group on.

Inactivate a reason to stop new cancellations from using it. Orders already cancelled keep the reason they were given.

Ordering locations

Define wards, operating rooms, the emergency department, and other places orders come from.



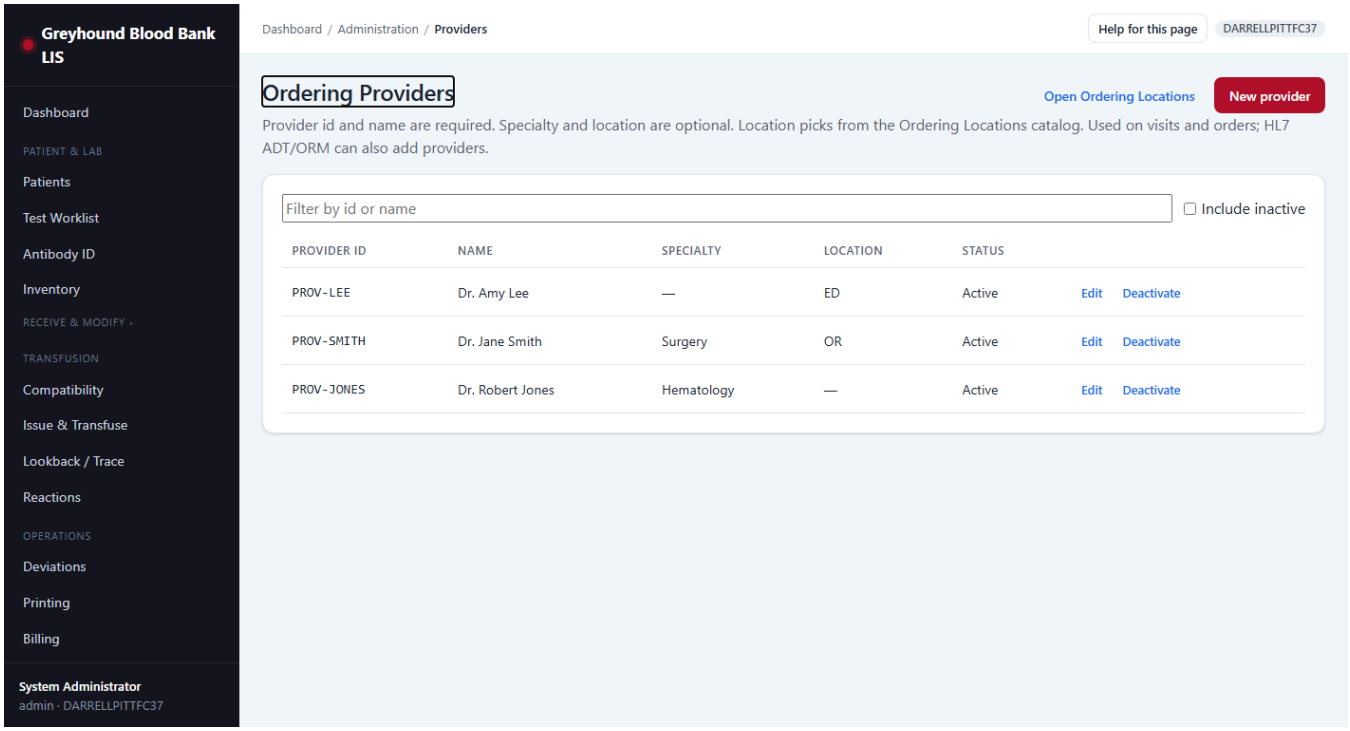
Ordering Locations are the wards, operating rooms, emergency department, and other sources an order or a visit can name. Code is required. Name and department are optional but are what people read, so fill them in.

Issue documentation also names a location. Using the same codes keeps the issue destination and the ordering ward comparable. If the interface sends a location code, it must match a row here or be translated. An unknown code fails the message rather than creating a location automatically.

Inactivate a closed ward. Do not delete it if visits already reference it.

Ordering providers

Define the clinicians who place orders or attend visits.



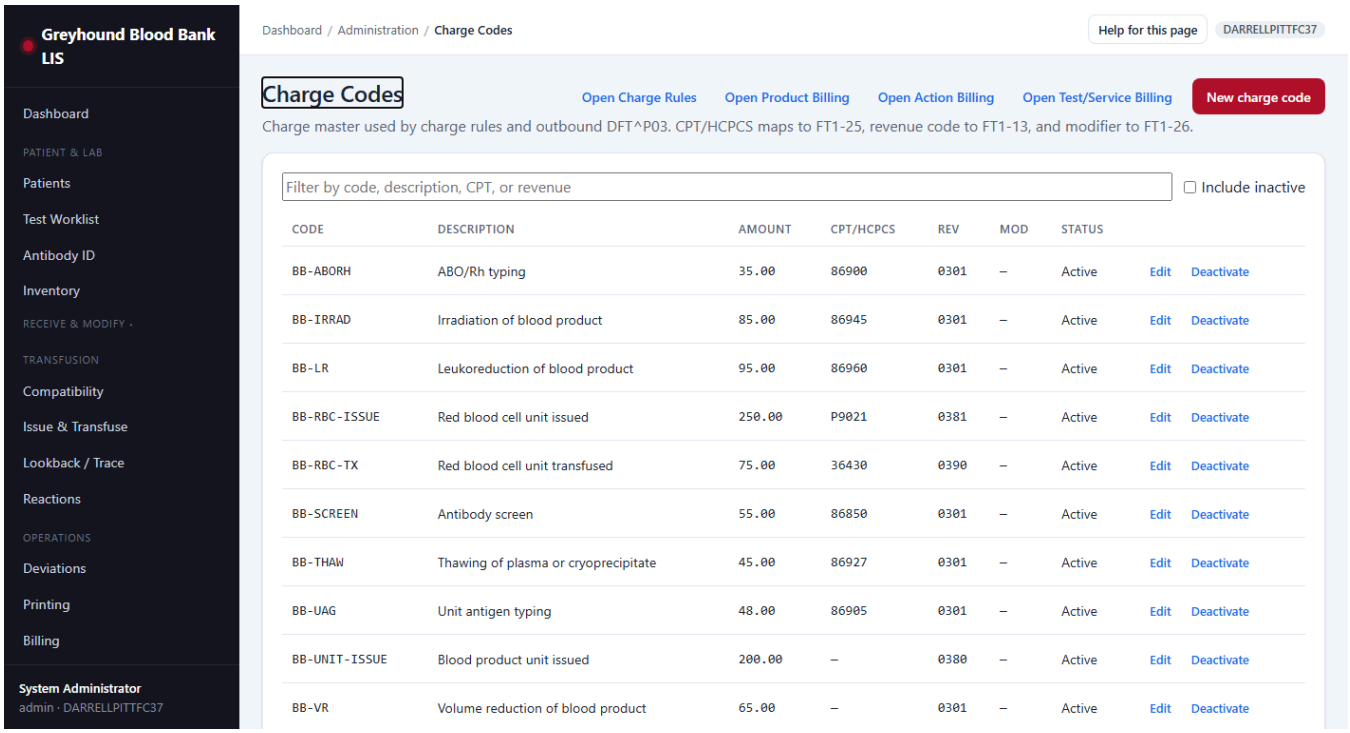
Ordering Providers are the clinicians stored on visits and orders. Provider id and name are required. Specialty and location are optional.

Interface orders match the ordering provider by the id in the message, after translation. A provider missing from this list causes the order message to fail or to arrive without a provider, depending on the map. Add the provider rather than substituting a generic "staff" provider, which makes every order look the same in the audit trail.

Inactivate a provider who has left. Historical orders keep the name that was current when they were placed.

Charge codes

Maintain the charge master: internal code, description, default amount, and optional CPT mapping.



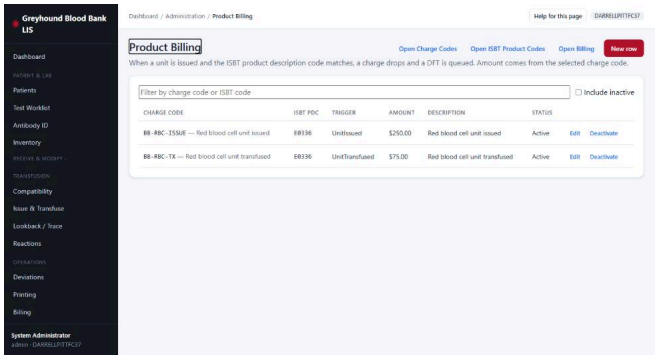
Charge Codes is the charge master. Each row has an internal code, a description, a default amount, and an optional CPT mapping. Charge rules, product billing, test billing, and action billing all point at these codes. They do not invent amounts of their own except where a product or test map supplies an override.

Changing an amount changes the next capture. It does not reprice charges already captured. Cancel and recapture is a billing action with a reason, not an edit on this row that rewrites history.

Retire a code by inactivating it once rules no longer point at it. A rule that still points at an inactive code will fail at capture. Update the rule first.

Product billing

Map an ISBT product code to a billing code and to the event that captures the charge, either issue or transfusion.



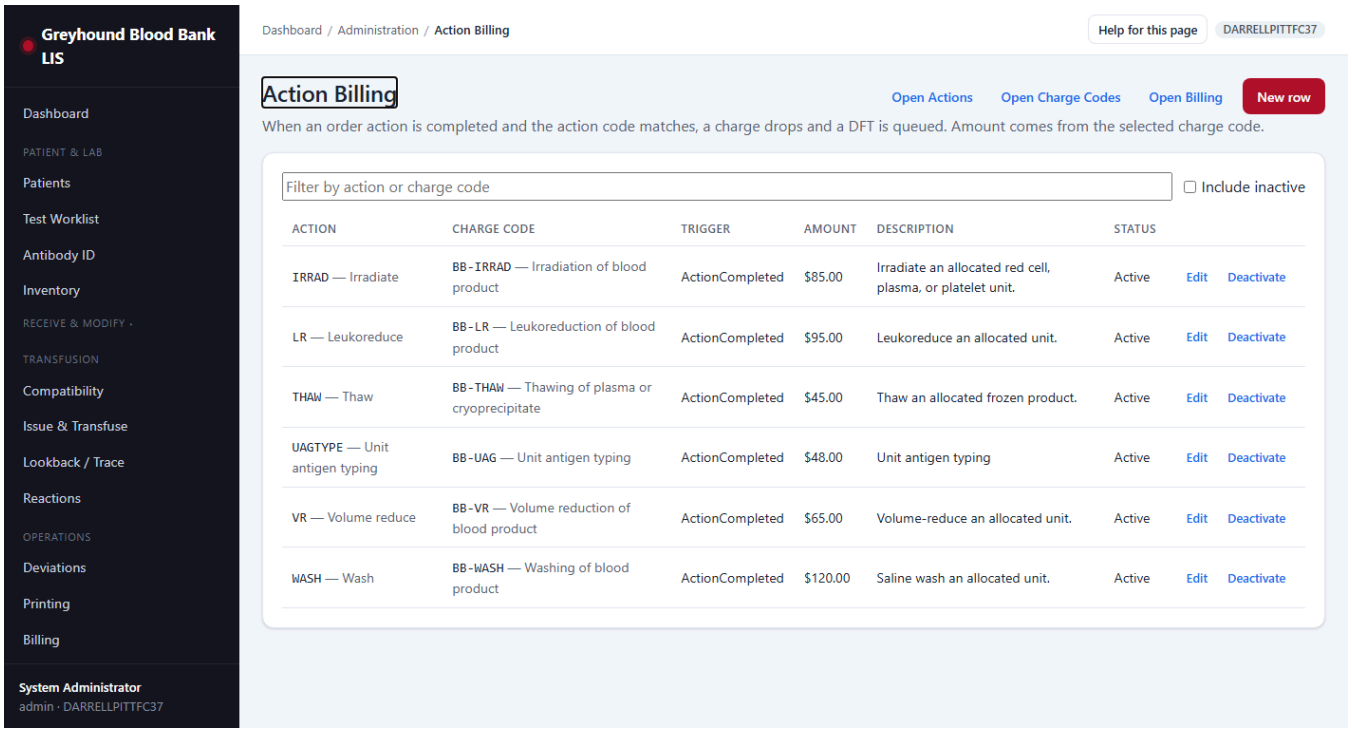
Product Billing maps an ISBT product code to a charge code. You choose whether the charge is captured when the unit is issued or when transfusion is documented. Optional price overrides the charge master's default for this product.

A product with no row is not charged. That may be correct for a product you do not bill, and it is a silent gap if you simply forgot. After adding a code to the ISBT catalog, check this map before the first issue.

The bench does not see this page. They see the captured row on the billing queue after the triggering event. Wrong trigger (issue versus transfused) is the usual reason a charge appears too early or not at all. Change the trigger here. Do not ask the bench to key the charge by hand.

Action billing

Map a completed modification action to a charge code. Completing the action queues the charge.



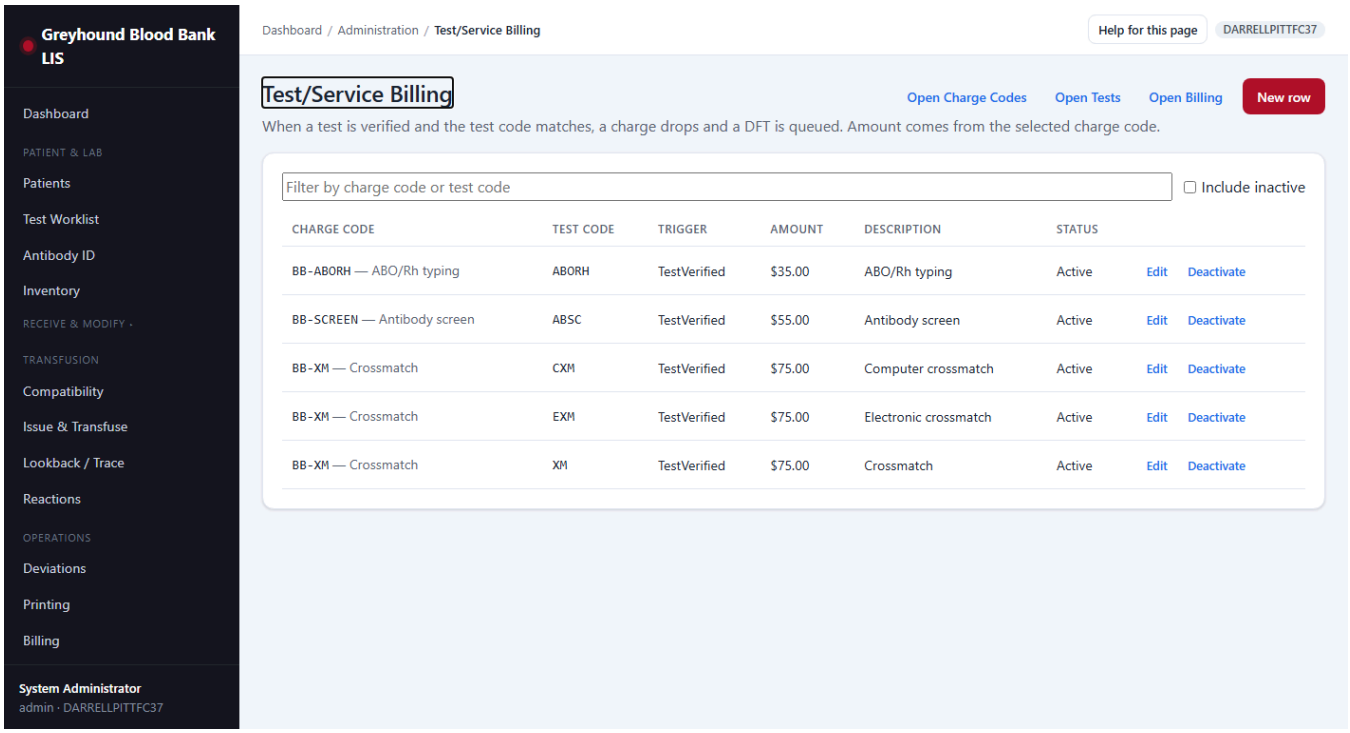
Action Billing maps an action to a charge code. The charge is captured when the action is completed, and a DFT is queued with the other outbound billing messages.

Map an action only if completing it is a separate billable event from issuing the product. Mapping both, when your payer considers the modification part of the product, double-charges. Leave the map empty when the product charge is enough.

The action must exist before you can map it. The charge code must exist on the charge master. An incomplete map does not warn the technologist at the moment they irradiate a unit. Check a test completion on a training database before you rely on the map.

Test and service billing

Map a test code to a charge code captured when the result is verified.



Test/Service Billing maps a test code to a charge code. The charge is captured when the result is verified, not when it is entered. Optional price overrides the charge master.

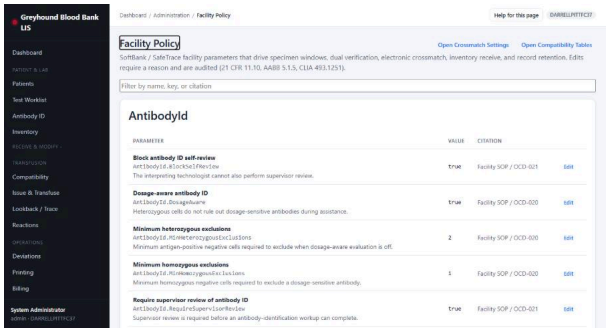
A test with no row produces no charge. Groupers bill through their member tests, so map the members. Mapping only the grouper code will not fire if the verified rows are the members.

Invalidating a result does not automatically cancel the charge. The billing queue is where a charge is cancelled, with a reason. Build that step into the procedure if a corrected test should not remain billed.

Rules and safety

Facility policy

Set specimen windows, dual verification, electronic crossmatch, inventory receipt, and retention. Each edit requires a reason.



Facility Policy holds the parameters that drive clinical behavior: how long a specimen is valid, when a second person must verify, whether electronic crossmatch is allowed, how inventory receipt works, and how long records are retained. The page groups them by category. Filter by name, key, or citation.

Each row shows the current value and the citation the parameter was written against. Select the row to edit. Boolean parameters are true or false. Numeric parameters show the allowed range. A value outside the range is rejected. Every save requires a reason.

> Facility policy. Electronic crossmatch stays off until you set the policy that allows it. Turning it on does not by itself make any patient eligible. Antibody history, open workups, and the minimums in Crossmatch Settings still apply.

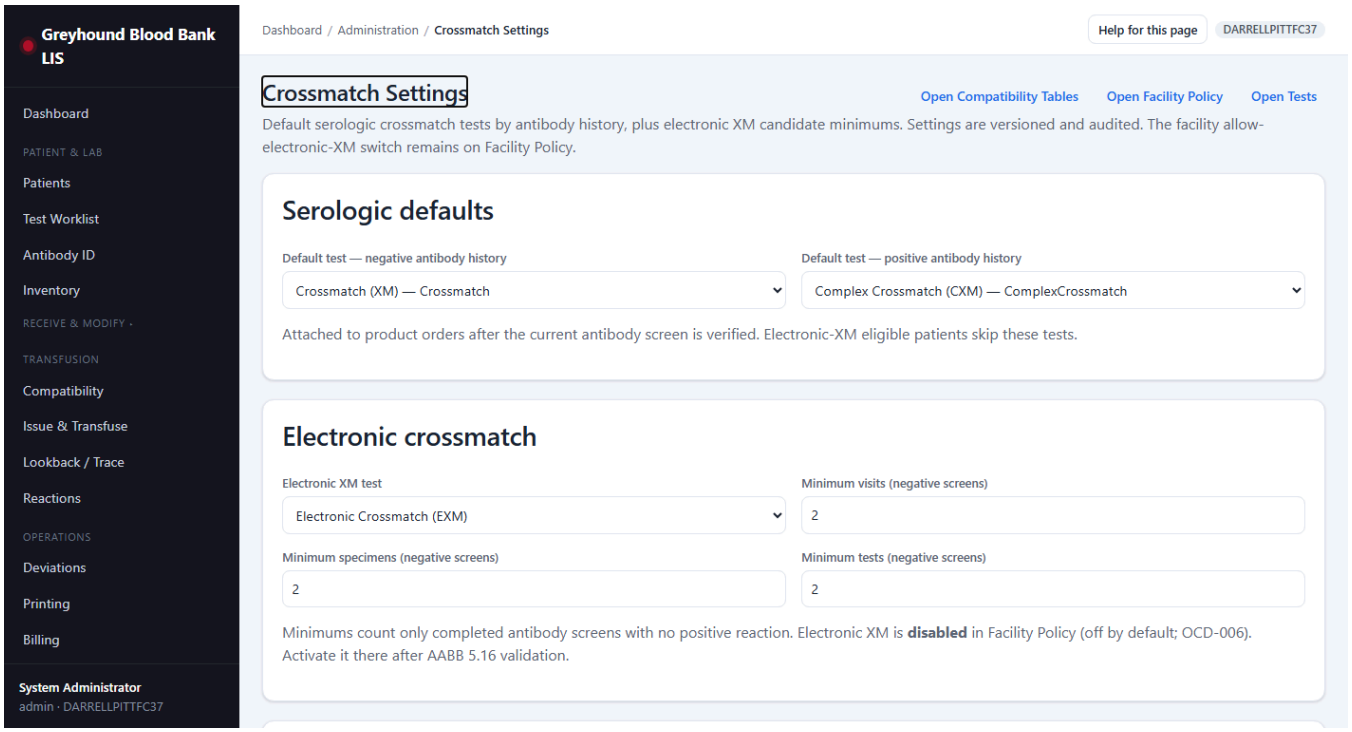
A parameter marked legal hold should not be edited to satisfy a convenience. The badge is there so you see the hold before you save.

Links on this page open Crossmatch settings and Compatibility tables. Those are separate catalogs. A change here does not edit the ABO table.

The bench feels a policy change on the next action. It does not reprint old issues. Say in the reason which procedure version you are implementing.

Crossmatch settings

Choose the default crossmatch tests and the number of negative screens required before electronic crossmatch can be considered.



Crossmatch Settings has two jobs.

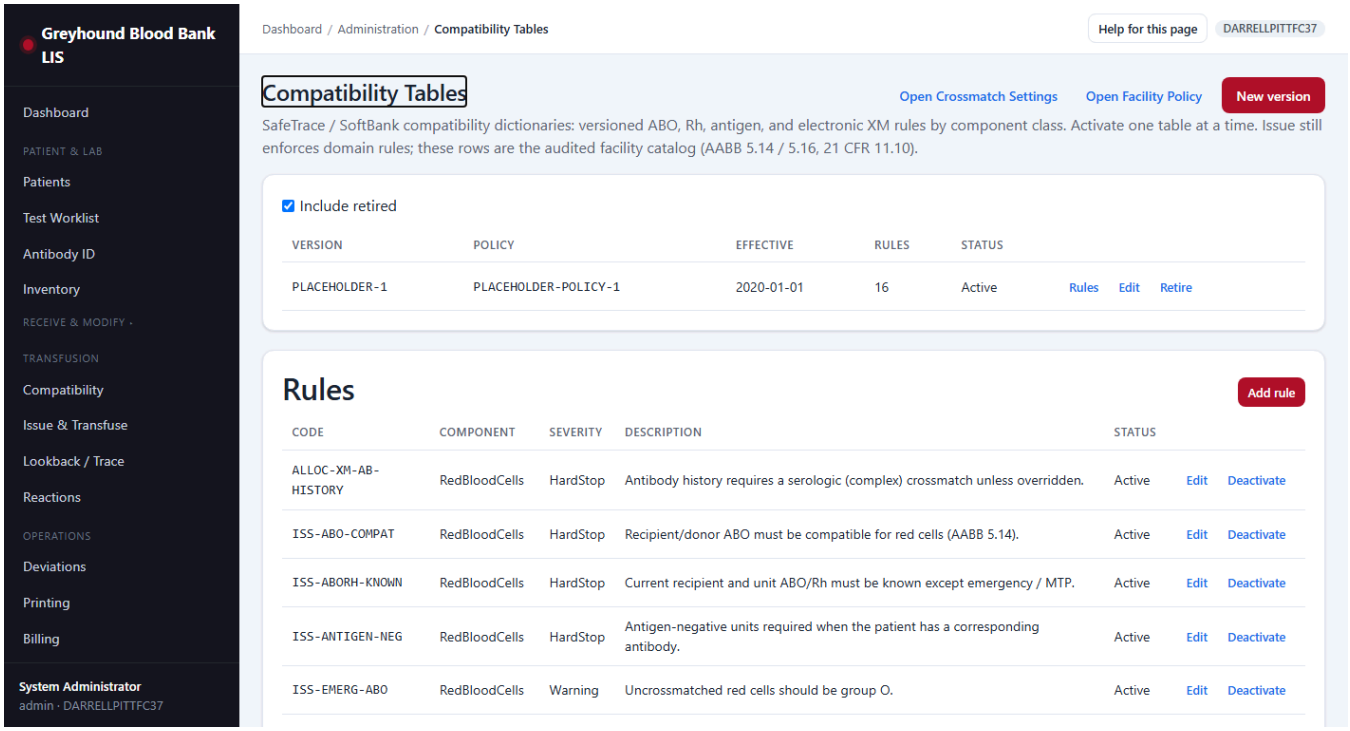
First, it selects the default crossmatch and computer-crossmatch tests the compatibility screen offers, including which test is used when the patient has antibody history. Point these at real test definitions. A default that names a retired test leaves the bench with an empty choice.

Second, it sets the minimum number of negative antibody-screen visits, specimens, and tests required before electronic crossmatch is eligible. These minimums apply only after Facility policy has allowed electronic crossmatch at all. Raising a minimum makes fewer patients eligible. Lowering it makes more patients eligible. That is a medical-director decision. Record it as such.

> Facility policy. Any antibody history row, including one that was later deactivated, still blocks electronic crossmatch no matter what these minimums say. The minimums do not override history.

Compatibility tables

Versioned ABO, Rh, antigen, and electronic-crossmatch rules by component class. Medical-director review is required.



Compatibility Tables are the ABO, Rh, antigen, and electronic-crossmatch rules for each component class. The issue gate reads the active version. A unit that the table calls incompatible is a hard stop, not a warning you can sign.

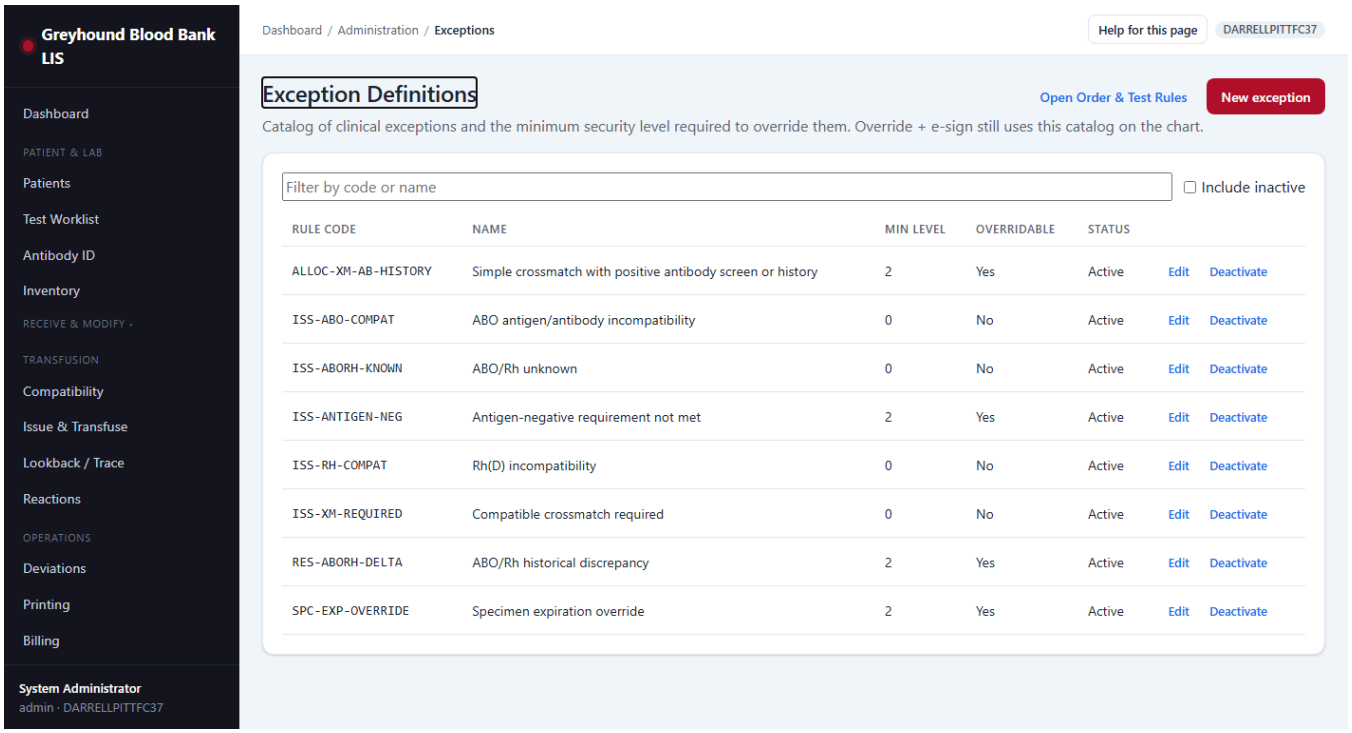
Edit a table only under medical-director review. The screen versions the change. The previous version remains available in history so you can see what was in force when an earlier unit was issued. Do not "fix" a historical issue by editing the table that was active that day. The issue stored the decision it made at the time.

Component class matters. A rule written for red cells is not automatically the rule for plasma. Check the class on the row before you save.

Electronic-crossmatch rows in this table do not turn electronic crossmatch on. Facility policy does that. These rows say which ABO and Rh pairs are acceptable when electronic crossmatch is already allowed.

Exceptions

Set the minimum security level required to override each clinical warning.



Exceptions lists clinical warnings that can be overridden and the minimum security level required. A user needs both the relevant permission (such as issue override or emergency release) and a role whose security level meets this minimum.

Raising the level stops a role that used to override the warning. Lowering it lets a lower level override. Do that only when the medical director accepts the risk. The reason you type is the change-control note.

A hard stop is not in this catalog as something you can soften. If a check has no override, it is not an exception. Do not look for a row that turns a hard stop into a warning. Emergency release is a different issue type, described in the clinical guide, not a switch on this page.

Order and test rules

Write versioned rules that decide when an order may be placed or a result may be verified. Open Help on that screen for the attribute catalog.



Order and Test Rules are conditions you write. When the condition is true, the action runs. Order rules see the patient and the order and run when an order is placed or changed. Test rules see those plus the result and run when a result is verified. An empty condition never fires.

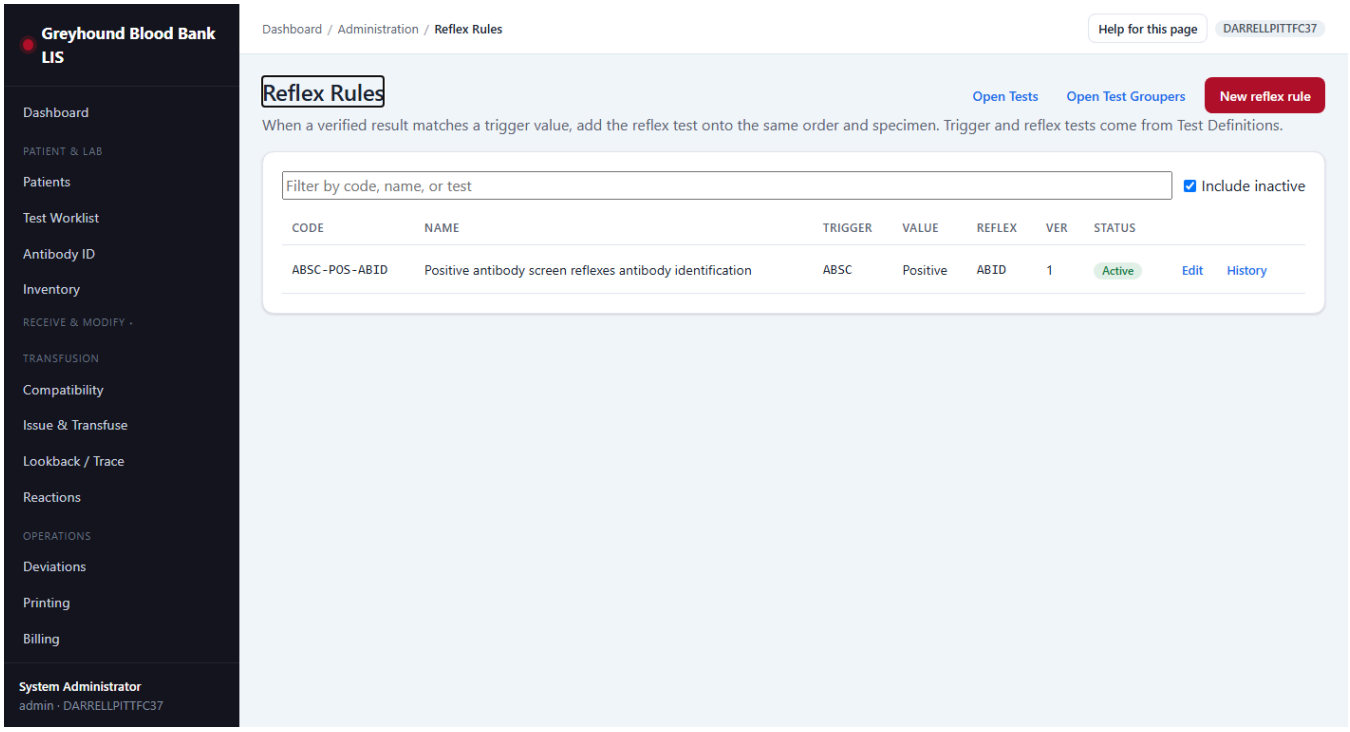
Use **Help** on this screen for the attribute list, the operators, and the functions. That panel is generated from the rule catalog, so it matches the engine. This topic does not duplicate that reference.

Rules are versioned like the other catalogs. A rule that blocks an order will block it for every user, including an administrator, unless the action itself is a warning with an override. Test the rule on a training patient before you activate it. An active rule with a typo can stop all ordering of a test.

> Warning. A rule is not a substitute for the issue gate. You cannot write an order rule that issues an incompatible unit, and you should not write one that tries to skip a hard stop. The gate still runs at issue.

Reflex rules

When a verified result matches a trigger, add another test on the same order and specimen.



Reflex Rules add a test when a verified result matches a trigger value. The added test is placed on the same order and specimen. It shows up on the worklist like any other unordered-until-now test.

Choose the trigger test, the value, and the reflex test. A trigger that is too broad (any result, or a value the test rarely uses) will add the reflex test on almost every verification. Confirm the value against the test definition's result codes, not against a nickname.

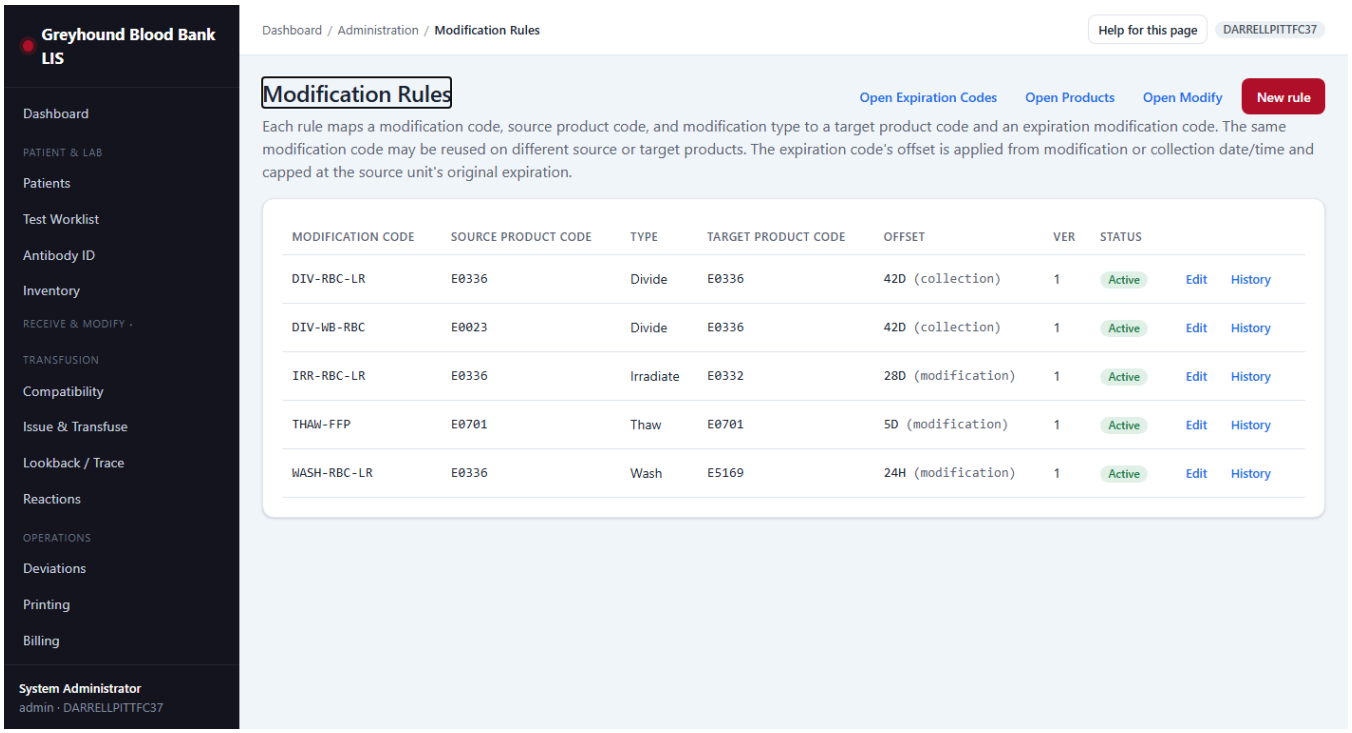
The reflex fires at verification, so an unverified entry does not add the test yet. Invalidating the original

result does not automatically cancel the reflex order. Cancel that order with a reason if it should not be done.

Reflex rules are for testing. They do not allocate a unit.

Modification rules

Define pool, divide, irradiate, wash, and the other modifications, including the product code of the resulting unit.



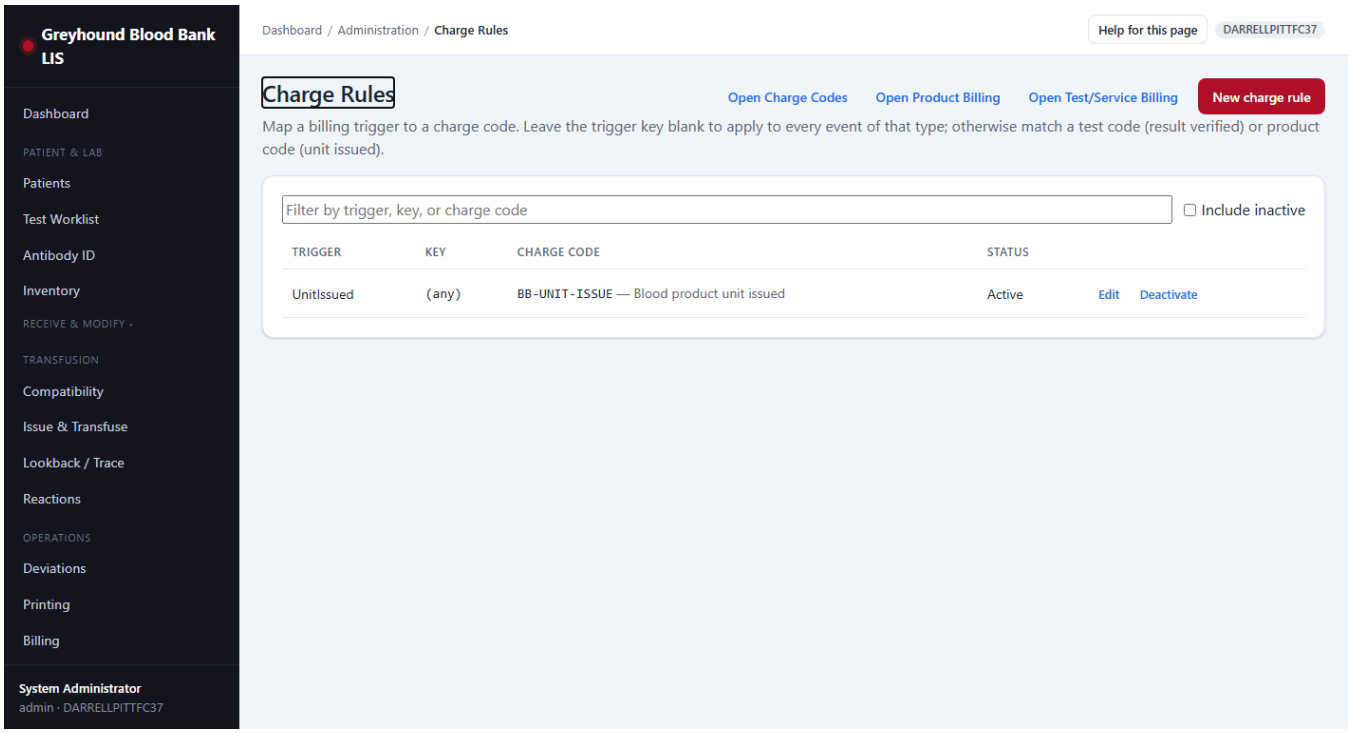
Modification Rules define component processing: which source products can be pooled, divided, irradiated, washed, or otherwise modified, and what product code the resulting unit receives. The bench screen Modify products only offers rules you activate here. A source or target choice keeps the product code. When the product name is blank, the choice says Product and the id. The code is not shown as the name.

Resulting units start in quarantine. Build the release into the procedure. A rule that produces an unknown ISBT code will fail at the moment of modification, not when you save the rule. Confirm the code exists in the ISBT catalog first.

Expiration changes come from the expiration-modification codes. If irradiation should shorten dating, the rule and the expiration code have to agree. A rule that keeps the original frozen expiration on a thawed product will offer an unsafe date at issue. Check a training modification and read the child unit's expiration before you activate the rule.

Charge rules

Map result verification and unit issue events to charge codes, optionally limited to one test or product.



Charge Rules map a clinical event to a charge code. The event is result verification or unit issue. An optional key limits the rule to one test code or one product code. A product choice keeps that product code. When the product name is blank, the choice says Product and the id. The code is not shown as the name. A rule with no key matches every event of that type, which will double-charge if a more specific map also exists.

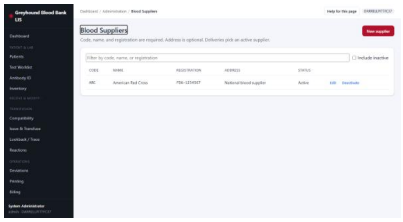
Prefer the specific maps on Test and service billing, Product billing, and Action billing for those cases. Use a charge rule when the event does not fit those three screens. Read the queue after a test event and confirm you captured one charge, not two.

Deactivate a rule rather than deleting it once it has captured charges. The captured rows keep the code they were given.

Inventory and ISBT

Blood suppliers

Name the suppliers that appear on deliveries. Code, name, and registration are required.



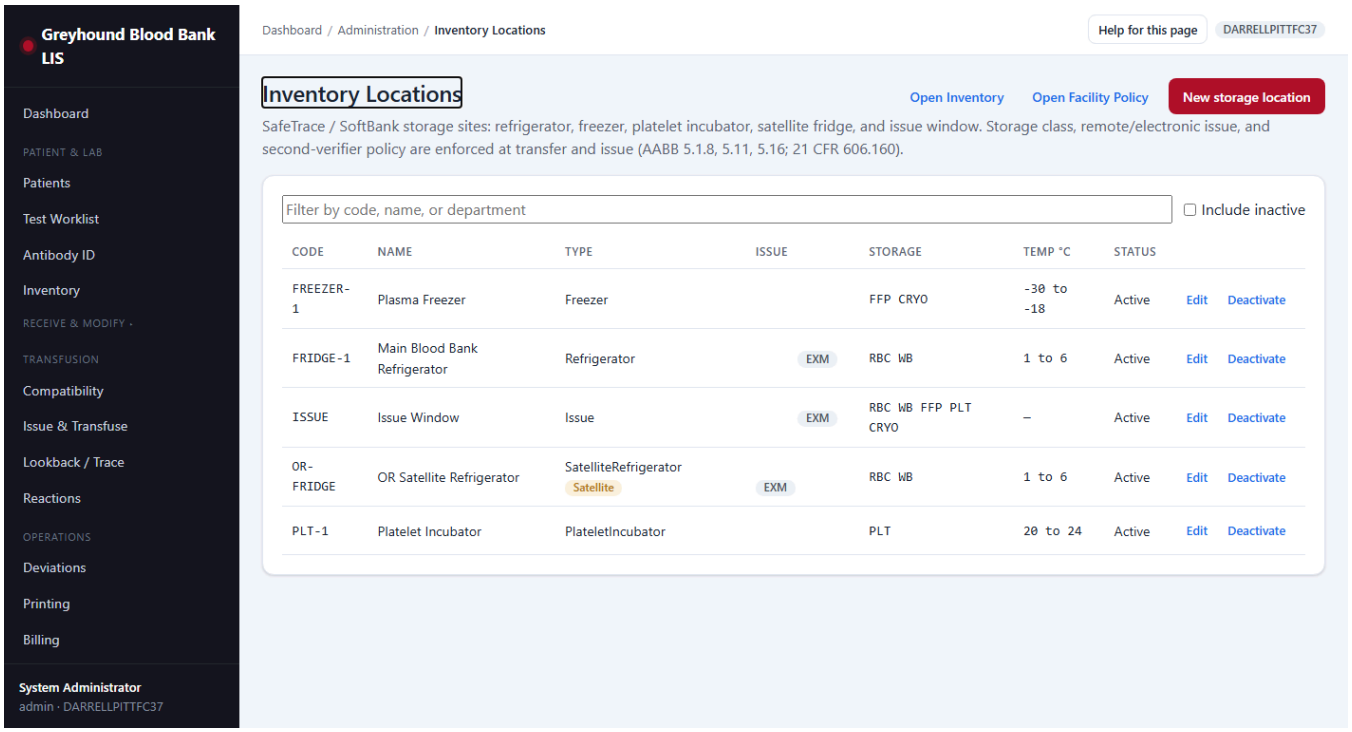
Blood Suppliers are the shippers you can select on a delivery. Code, name, and registration are required. Address is optional. A delivery cannot name a supplier who is not on this list, so add the supplier before the first shipment arrives.

Registration is the identifier your procedure uses for that supplier. Do not put a nickname in the registration field and the license number only in a comment. Lookback and receipt reports use the registration.

Inactivate a supplier you no longer receive from. Historical deliveries keep the supplier they were received under.

Inventory locations

Define storage sites, satellite refrigerators, and remote-issue locations, including storage-class rules.



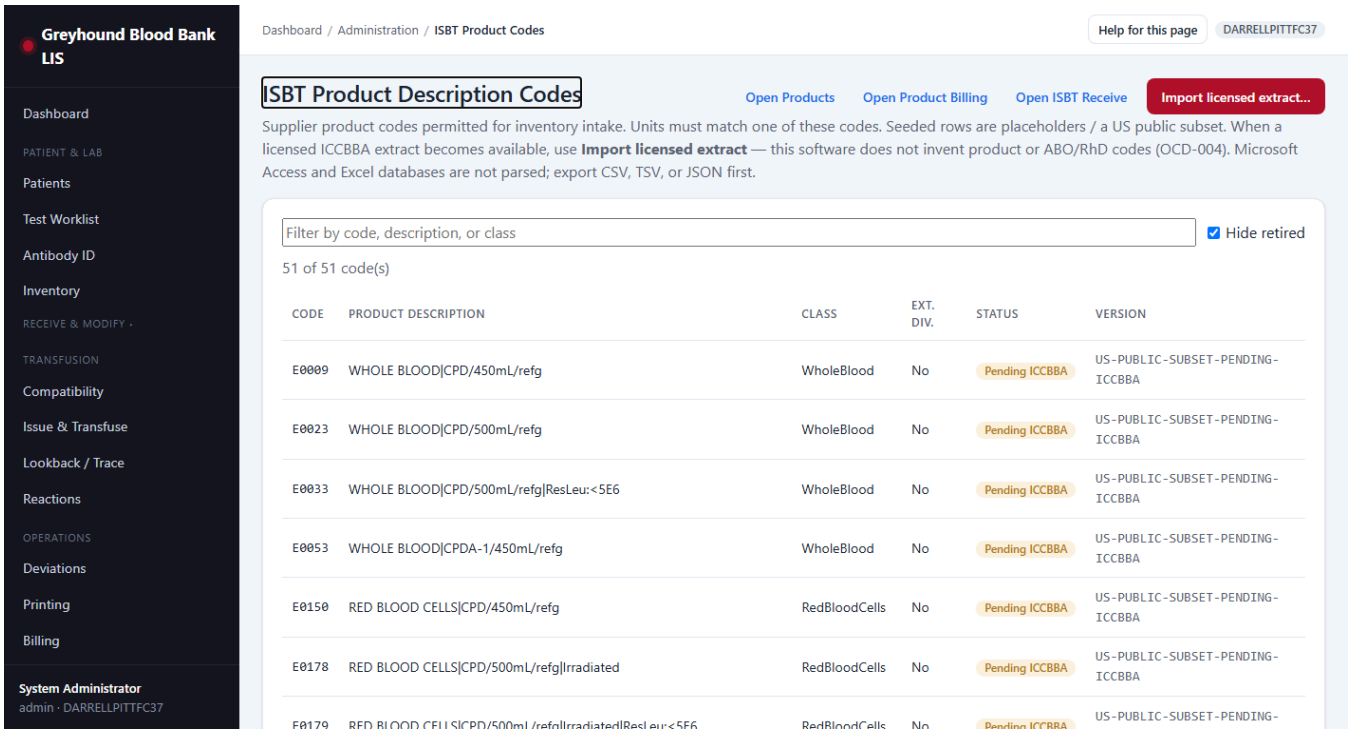
Inventory Locations are refrigerators, freezers, platelet incubators, satellite storage, and remote or electronic-issue sites. The unit chart and the inventory list show the location name. The issue gate also reads the location's storage class when it decides whether a unit may be issued from that site.

Create a location before you transfer a unit there. A transfer to an unknown place is not a location. Mark remote-issue locations explicitly so electronic or remote issue rules apply only where you intended.

Inactivate a location that is out of service so new transfers cannot select it. Units still sitting there remain visible until you transfer them.

ISBT product codes

Review the product-code catalog and import a licensed ICCBBA extract when your facility has one.



ISBT Product Codes is the catalog ISBT receive, modification, and product billing match against. The rows shipped with the application are a placeholder subset, not the licensed ICCBBA tables. Official codes are not included.

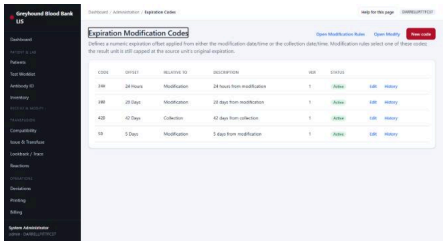
If your facility holds an ICCBBA license and you have an extract in CSV, TSV, or JSON, import it from this page. Review the import before you rely on it for live receipts. A code that is missing causes receive and modification to reject the unit. Do not invent a look-alike local code for a licensed product.

> Facility policy. Using ISBT 128 product codes requires the facility's own ICCBBA license. Importing an extract you are not licensed to use is not a configuration shortcut.

After new codes are imported, check Product billing and Modification rules so the new codes charge and modify the way you intend.

Expiration codes

Define ISBT expiration-modification codes that change dating after irradiation, pooling, or similar processing.



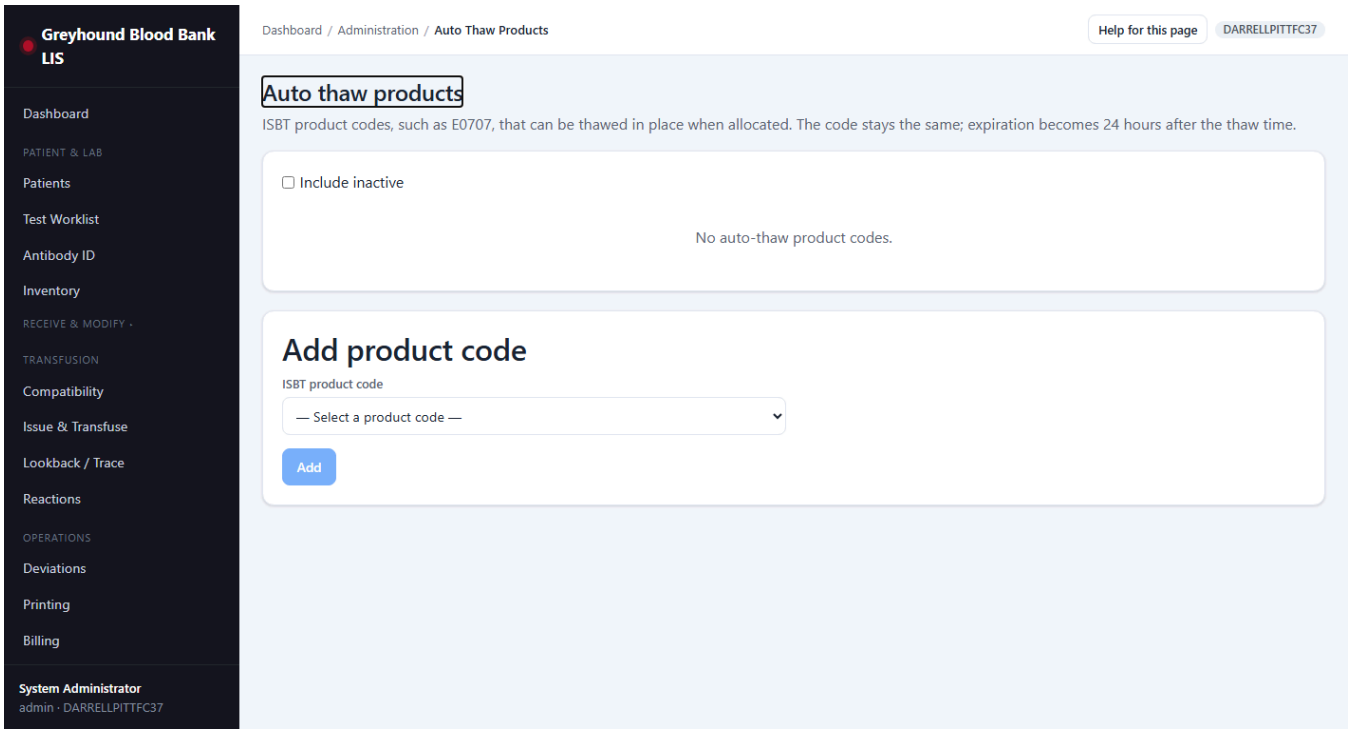
Expiration Codes are the ISBT expiration-modification codes applied when processing changes the dating, for example after irradiation or pooling. The modification rule points at a code. The code calculates the new expiration. The bench does not type a new date.

A wrong code leaves thawed or irradiated units with the original expiration, or shortens dating that should have been unchanged. Compare the calculated date on a training unit with your procedure before you activate the rule that uses the code.

These codes are part of the same licensing caution as the product-code catalog. Enter a code your license covers. Do not copy a table from an unlicensed source into this screen.

Auto thaw products

List ISBT plasma codes that may be thawed in place when they are allocated. Expiration becomes 24 hours from the thaw.



Auto thaw products lists ISBT plasma codes, such as E0707, that may be thawed in place when a technologist allocates them. The unit's expiration becomes 24 hours from the thaw time. Codes that are not on this list are not thawed automatically. They wait for an explicit modification.

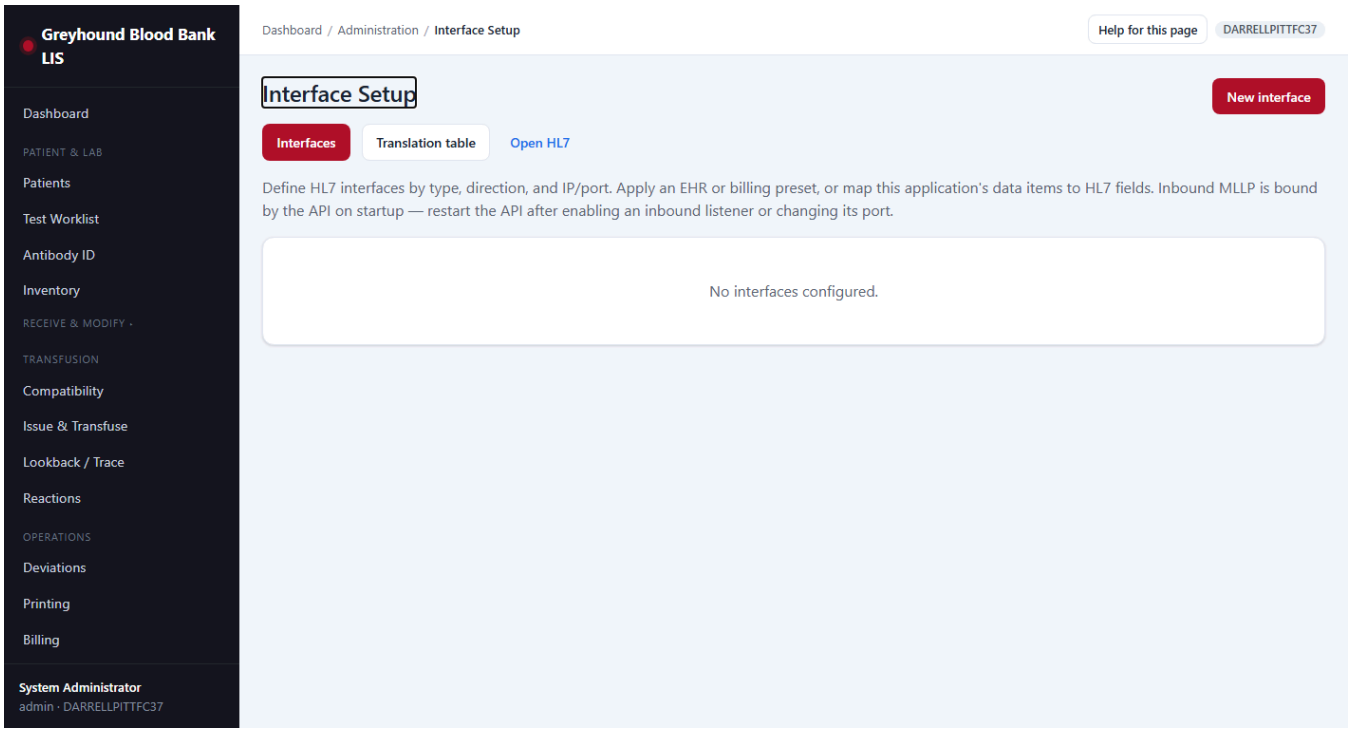
Add a code only if your procedure allows thaw-on-allocation for that product and the 24-hour expiration is the dating you intend. A code added by mistake will thaw a frozen unit the moment someone reserves it, and the original frozen expiration will no longer be the one on the label.

Remove a code to stop future auto-thaw. Units already thawed keep the thawed expiration. They do not revert to frozen stock.

Interfaces and printers

HL7 interface setup

Configure each HL7 interface: direction, address, vendor preset, field mapping, and where translations are edited.



Interface Setup defines each HL7 connection: the type of message, the direction, the address and port, a vendor preset when you start from one, and the field mapping. The operational log, where you replay a failed message, is the HL7 console in the clinical guide. This page changes what the connection does.

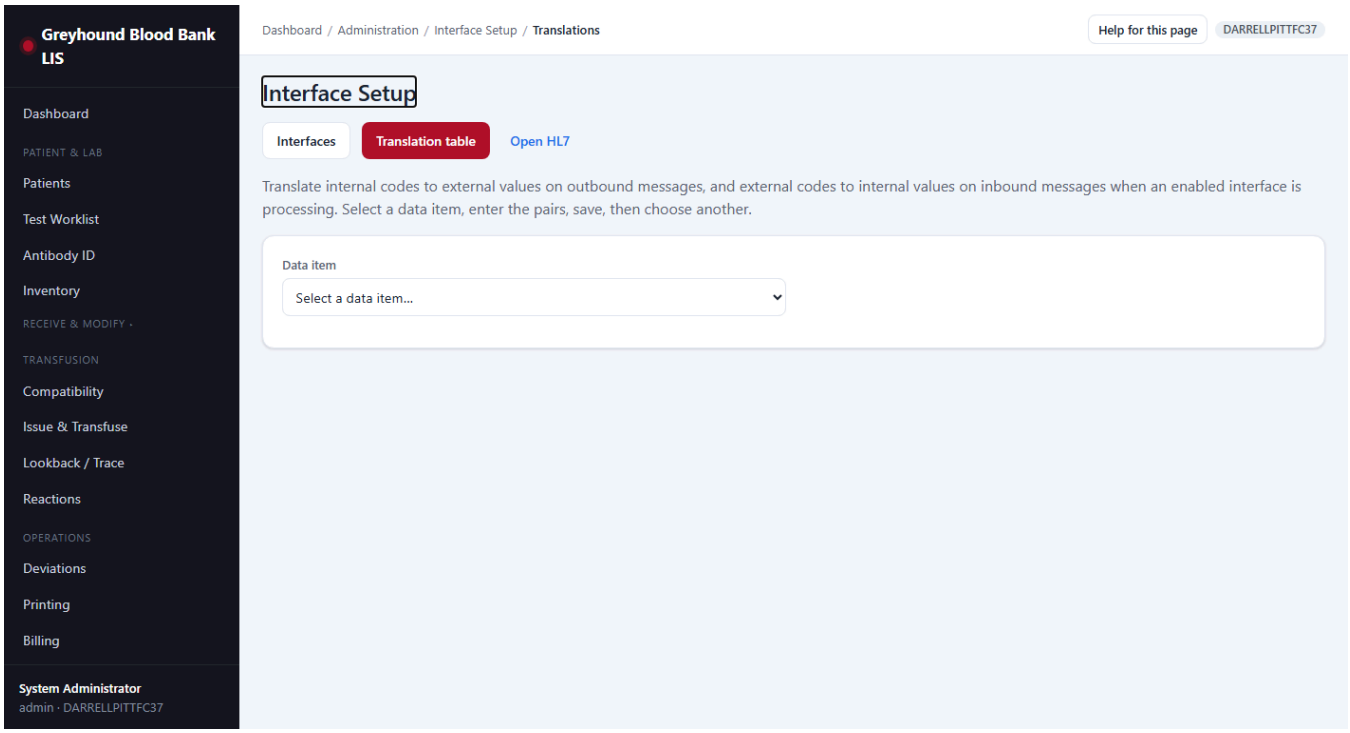
Edit a live interface only when you can watch the next message. A bad map files orders or results on the wrong field, or rejects every message. The translation table for coded values is Interface translations. A map that expects a translated code will fail until the translation exists.

> Warning. Activating an interface that points at a production host sends and accepts real messages. Confirm the host and the port before you activate. A training system must not point at the clinical interface engine.

Vendor presets are a starting map, not a validation. Compare the preset with the specification you received from the sending system before you go live.

Interface translations

Map incoming coded values, such as location or provider codes, onto the codes this system stores.



Interface Translation Table maps a value in an inbound message onto a value in this system. Typical rows translate a location code, a provider id, a specimen type, or a test code. The HL7 setup names which fields use the table.

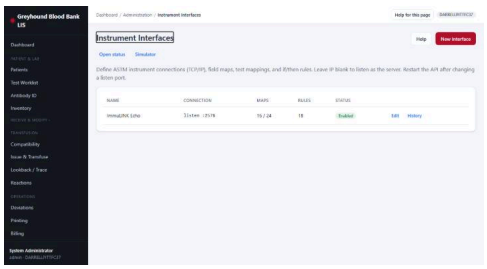
An untranslated code does not create a new location or provider. The message errors, and the order or result does not file. Add the translation, then replay the message from the HL7 console. Replaying before you add the row files the same error again.

When the sending system changes a code, add the new translation and leave the old one if historical replays still need it. Deleting the old row makes those replays fail.

Do not translate two different incoming codes onto one local patient or location unless they are truly the same place. That is how two wards become indistinguishable on the chart.

Instrument interface setup

Configure ASTM instrument connections, field maps, test mappings, and if/then message rules. Open Help on this screen for the catalog.



Instrument Interfaces in Administration is the configuration for each analyzer connection. The status log that shows whether messages are arriving is the operational Instrument interfaces screen. The simulator sends a test message.

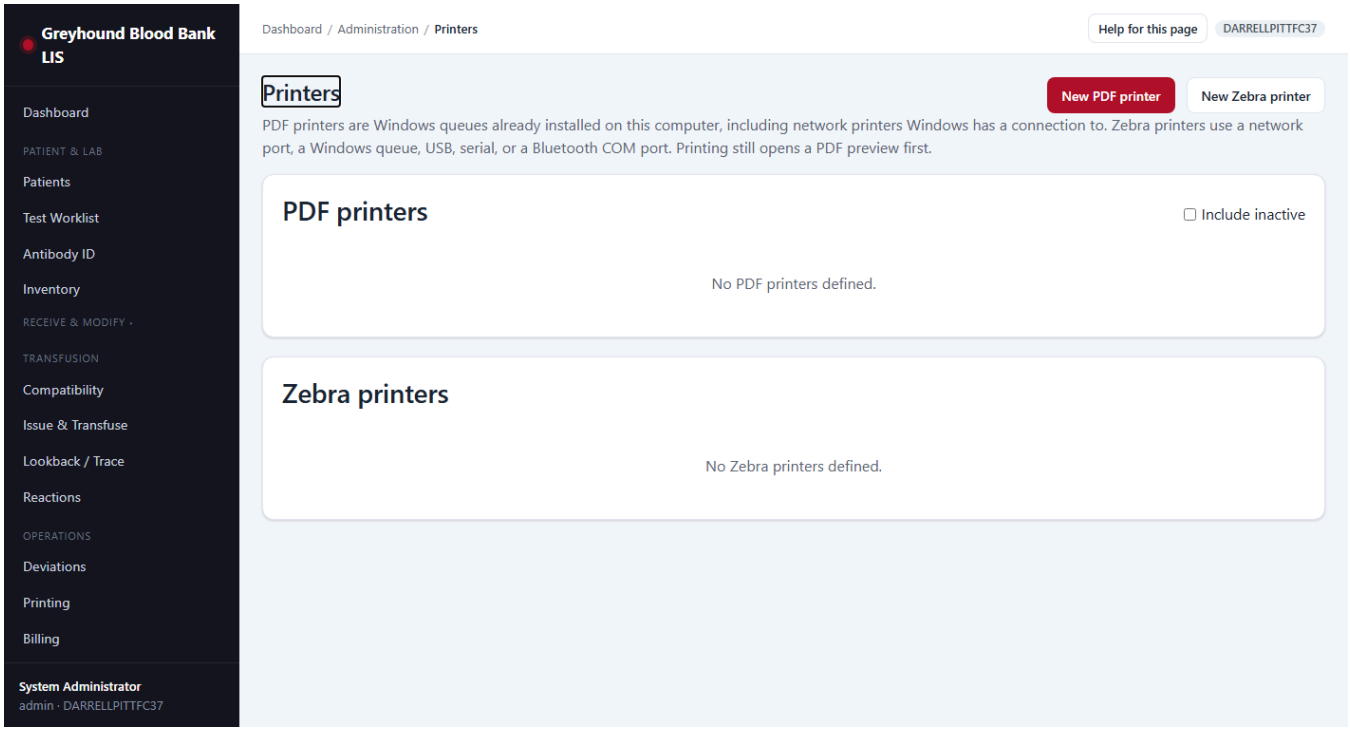
On this page you set the connection, the field map, which instrument test code becomes which LIS test, and if/then rules that adjust a message before it files. **Help** on this screen lists the instrument catalog fields. Use that panel while you are editing. It stays current with the catalog.

A test code that is not mapped does not file a result. The message appears as an error on the operational log. Map the code, then decide whether to replay. Do not map two instrument codes to the same LIS test unless both codes really are that test. You will verify indistinguishable results.

> Warning. A mapping change affects the next message from that instrument, including a live analyzer. Prove the map with the simulator on a training accession first.

Printers

Register PDF queues and Zebra printers by network, USB, Windows driver, serial, or Bluetooth.



Printers registers the queues the print screen can send to. PDF queues are the printers installed on this computer for preview and paper. Zebra printers can be network, USB, Windows driver, serial, or Bluetooth. A printer that is not registered here does not appear when a technologist prints a label or a tag.

Test a new printer with a specimen label and a P-tag before you tell the bench to use it. Label stock and tag stock are different sizes. A queue pointed at the wrong device prints a specimen label on a tag or

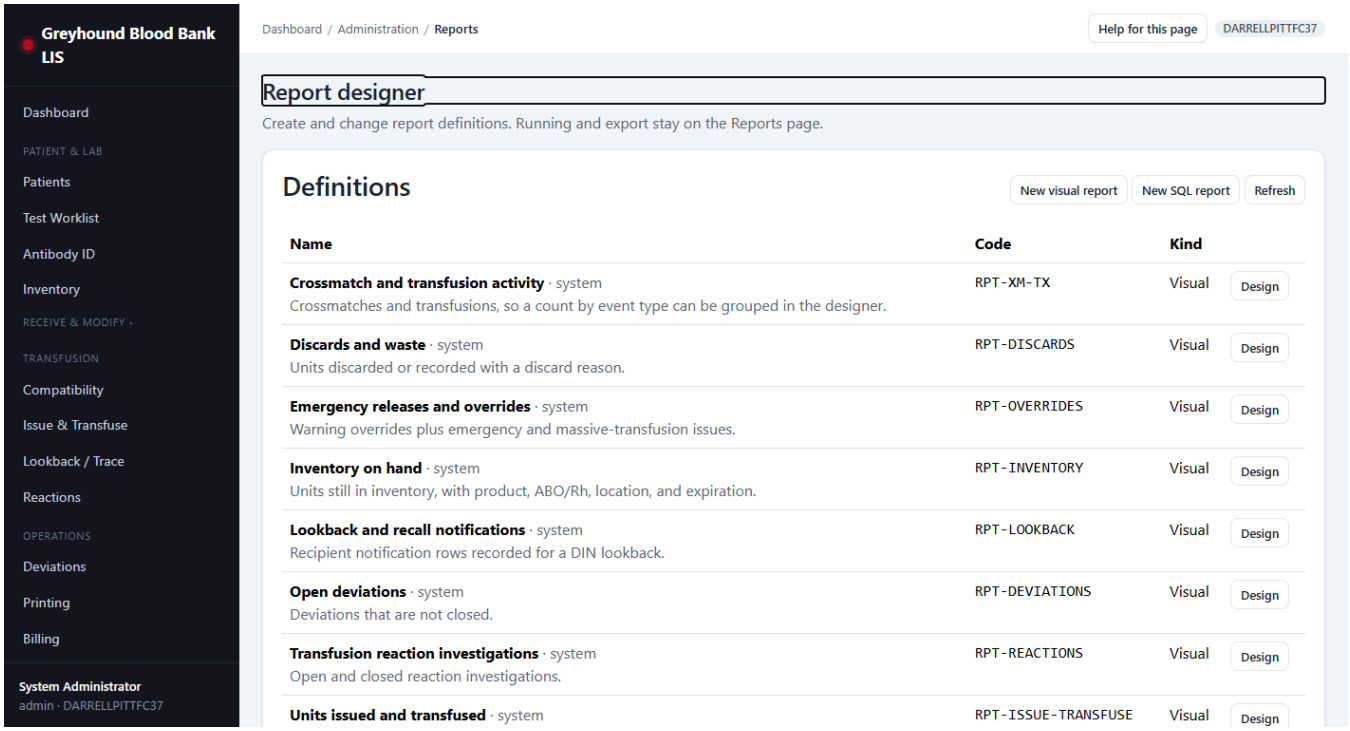
sends ZPL to a PDF driver.

This page does not design the label. Layouts are Specimen label and P-tag layout. Removing a printer stops new jobs from selecting it. It does not recall labels already on the bench.

Reports

Report catalog

Open the list of report definitions. Running a report stays on the Reports screen in the clinical guide.



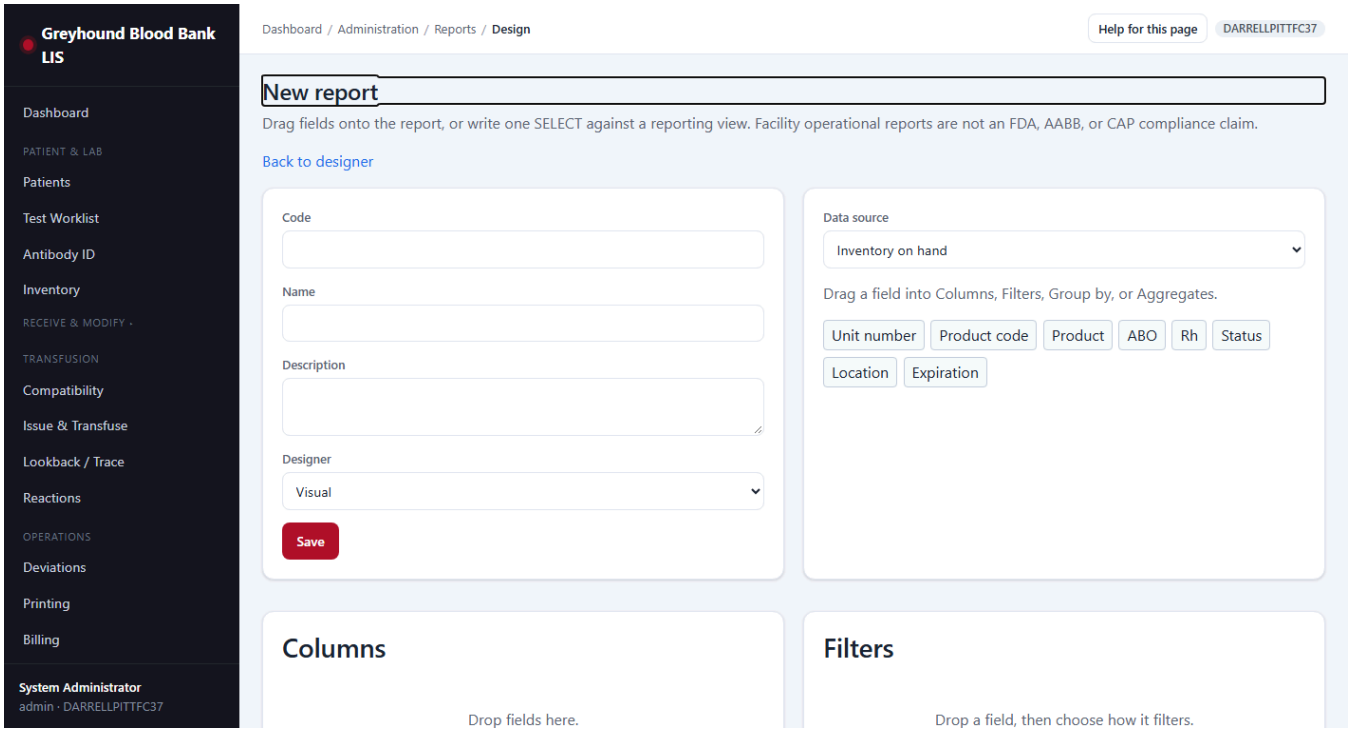
Report designer in Administration opens the catalog of definitions. Running and export stay on Reports in the clinical application, and they require run permission rather than design permission. A user who can design can change what every runner sees. Limit design permission to people who should change published reports.

Open a report to edit it, or create one. Unfinished drafts should stay unpublished so the run list does not offer a query you have not checked. A published report with a broken query fails at run time for the technologist. Run it yourself after you publish.

Deleting or replacing a published report does not delete files people already exported. It changes the next run.

Report designer

Build a visual report or a SQL report. SQL help on this screen lists the views and functions you can use.



The designer builds one report. Visual mode places columns from the reporting views. SQL mode writes a query. **Help** in SQL mode lists the views and functions that are allowed. Use those. The application does not give the report a direct query against every table, and a query that does not use the published views will be rejected.

Parameters you add become fields on the run screen. Name them so a technologist knows what to type. A parameter called "date" with no indication of collection versus issue will be filled in differently by every shift.

Save, then run the report from the clinical Reports screen with a range you understand. Confirm the row count before you tell anyone the report is published. SQL that returns patient identifiers is still a patient report. Do not publish a query that selects more identifiers than the procedure allows.

> Warning. A SQL report runs with the application's reporting access. It can return every patient who matches. Treat the definition as carefully as a clinical extract.